

ABFM Question Bank

Outdated & Superseded Questions — Removal Log

This document lists 361 questions removed from the ABFM question bank because their correct answers reflect outdated, superseded, or revised clinical guidelines. Each entry includes the question number, exam year, and the current recommendation that makes the original answer incorrect or misleading.

Year	Questions Removed
2015	58
2016	65
2017	60
2018	50
2019	47
2020	45
2021	36
Total	361

2015 Examination — 58 questions removed

Q4

Haloperidol's role in bipolar maintenance has evolved. Some atypical antipsychotics (including newer agents) are now recognized maintenance options, and the list of approved maintenance therapies has expanded significantly since 2015. More importantly, haloperidol is not a standard maintenance agent but some guidelines now recognize certain antipsychotics more broadly; the question framing may mislead students about current approved maintenance options.

Q8

Asthma stepwise management guidelines have been updated. This patient has moderate persistent asthma (symptoms >2 days/week, nighttime awakenings >1x/week). Current NAEPP/GINA guidelines recommend low-dose ICS as step 2, and low-dose ICS + LABA or medium-dose ICS as step 3. Additionally, current guidelines (2020 GINA) now recommend against scheduled SABA alone and favor ICS-containing regimens; the framing of 'medium-dose ICS' as first addition may not align precisely with current stepwise approach which starts with low-dose ICS.

Q10

Current ATA thyroid nodule guidelines (2015, updated and reinforced through 2023) recommend TSH as initial test, which remains correct, but the management of a 1 cm thyroid nodule has changed significantly — current guidelines suggest FNA may not be indicated for nodules <1 cm unless high-risk features are present on ultrasound. The question asks only about next step (TSH) which remains current, but borderline given evolving nodule management algorithms.

Q12

CDC/IDSA guidance on influenza chemoprophylaxis in long-term care facilities has been updated. Current guidance specifies oseltamivir prophylaxis for all residents regardless of vaccination status during an outbreak, but also includes nuanced recommendations about duration, staff prophylaxis, and the role of rapid testing. The answer 'D' (no chemoprophylaxis for vaccinated residents/staff) is clearly wrong per current guidelines, but option A as stated may not fully reflect current CDC outbreak response protocols which have been revised, and the question framing around H1N1 specifically is outdated as H1N1 is now the seasonal strain. The question could mislead students about current outbreak management protocols.

Q15

The AUA updated its definition of clinically significant microscopic hematuria in 2020 guidelines. The 2020 AUA guideline raised the threshold to ≥ 3 RBCs/hpf on a single properly collected specimen (replacing the older 2-of-3 specimens requirement), but more importantly the 2023 AUA microhematuria guideline further refined risk stratification and evaluation criteria. Students should be aware that the ≥ 3 RBCs/hpf threshold remains but the overall evaluation framework has been substantially updated, and teaching this question in isolation could reinforce outdated evaluation pathways.

Q17

The question flags clarithromycin as the answer to avoid in prolonged QT syndrome, but current guidelines would also flag moxifloxacin (a fluoroquinolone) as a major QT-prolonging agent that should be avoided. In fact, moxifloxacin carries a strong QT-prolongation warning and is considered higher risk than clarithromycin by many current references. A student today might be misled because moxifloxacin is also a correct answer to avoid, making the 'only clarithromycin' framing potentially harmful. Additionally, clarithromycin has received updated cardiovascular risk warnings since 2015 beyond just QT prolongation.

Q21

JNC 8 is no longer the current standard. The ACC/AHA 2017 hypertension guidelines replaced JNC 8 and changed BP thresholds (hypertension now $\geq 130/80$) and treatment recommendations. Beta-blockers being excluded as first-line is a JNC 8-specific teaching point that does not reflect current ACC/AHA 2017 guidelines, which do not explicitly exclude beta-blockers in the same way. Teaching JNC 8 specifically is outdated.

Q26

Data on leading causes of unintentional death in children has shifted. As of recent CDC data, drug poisoning/overdose has surpassed motor vehicle accidents as the leading cause of unintentional injury death in some pediatric age groups, and the rankings vary by age group. This question could reinforce outdated epidemiological facts.

Q31

Current guidelines (ACC/AHA, USPSTF) have evolved significantly regarding cardiovascular risk assessment. The correct answer of 'lipid profile' may be misleading in context — coronary artery calcium (CAC) scoring has risen substantially in prominence as a risk-refinement tool in current ACC/AHA guidelines for intermediate-risk patients. More importantly, the framing of the question around what to offer instead of a stress test in an asymptomatic patient now has updated USPSTF guidance (2018) that recommends AGAINST screening with resting or exercise ECG in low-risk asymptomatic adults, and the role of various tests has been recharacterized. The question could reinforce outdated risk stratification thinking.

Q34

Pediatric community-acquired pneumonia treatment guidelines have been updated. While amoxicillin remains first-line for typical bacterial pneumonia in children, the clinical scenario describes a 3-year-old with mild symptoms and low-grade fever, which in current practice (per IDSA/PIDS 2011 guidelines still current, but evolving stewardship principles) may not warrant antibiotics at all if viral etiology is suspected. The question does not clarify bacterial vs. viral pneumonia sufficiently, and current antimicrobial stewardship guidance emphasizes watchful waiting for mild pediatric respiratory illness. A student today could be misled into prescribing antibiotics for what may be a viral illness.

Q40

For symptomatic peripheral arterial disease, current guidelines (ACC/AHA 2016 and 2024 updates) have significantly elevated the role of dual antiplatelet therapy and specifically antiplatelet agents like clopidogrel, which may be preferred over aspirin alone in PAD. Additionally, the COMPASS trial (2017) established rivaroxaban plus aspirin as a recommended option for symptomatic PAD, changing the antiplatelet/anticoagulation landscape substantially. Aspirin alone as the answer may now be considered incomplete or suboptimal compared to current guideline-recommended options, potentially misleading students about current best practice.

Q45

The live attenuated intranasal influenza vaccine (LAIV) recommendation has changed significantly. The ACIP withdrew its preference for LAIV in children in 2016-2017 due to poor effectiveness data, then reinstated it as an option (not preferred) in 2018. Current CDC/ACIP guidance states LAIV4 is an acceptable option for non-pregnant individuals 2-49 years, but it is NOT preferred over inactivated vaccine in children 2-6 years. The answer 'It is more effective in children age 2-6 years than the inactivated vaccine' no longer reflects current ACIP recommendations and could mislead students about current vaccine preferences.

Q47

While benzodiazepines remain first-line for alcohol withdrawal, chlordiazepoxide specifically is less favored in current practice compared to other benzodiazepines (e.g., lorazepam, diazepam) especially in patients with liver disease or in outpatient settings. More importantly, carbamazepine is now recognized by ASAM and current guidelines as a valid and often preferred first-line alternative for mild-to-moderate alcohol withdrawal (outpatient setting), making the framing of this question potentially outdated. The ASAM 2020 clinical practice guideline on alcohol withdrawal management elevates carbamazepine as a first-line option, which could make answer B defensible in current practice.

Q54

Sepsis-3 (2016) redefined septic shock criteria: requires vasopressor need to maintain MAP ≥ 65 mmHg AND serum lactate > 2 mmol/L despite adequate fluid resuscitation. The question asks which finding 'confirms septic shock' and lists lactate of 2.0 mmol/L as correct, but under Sepsis-3, the threshold is > 2 mmol/L (not ≥ 2), and lactate alone is insufficient — vasopressor requirement is also needed. Additionally, the older SIRS-based sepsis framework used in this question's context has been replaced, making the diagnostic framing outdated.

Q59

Current ACIP and IDSA guidelines for asplenia do not recommend empiric antibiotics for 'any episode of fever' as standard practice for all asplenic patients beyond the pediatric/early post-splenectomy period. Updated recommendations emphasize vaccination (pneumococcal, meningococcal, Hib, influenza) as primary prevention. The Hib vaccine recommendation (option C) is actually part of current guidelines for asplenic patients who were not previously vaccinated or need revaccination, making option C potentially more correct in current practice. The 'antibiotics for any fever' answer reflects older, more broadly applied guidance that has been nuanced in current guidelines.

Q62

Pneumococcal vaccine schedules have significantly changed. Current ACIP recommendations use PCV15 or PCV20 for children, and the indications and schedules for PPSV23 second doses in children have been updated. The question references older pneumococcal polysaccharide vaccine (PPSV23) dosing logic that may not reflect current 2023-2024 ACIP guidance, which has shifted toward higher-valency conjugate vaccines as primary series with revised catch-up and high-risk schedules.

Q65

The blood pressure thresholds cited (142/90 and 138/88) reflect JNC 7 criteria. The 2017 ACC/AHA guidelines (adopted by AAFP) reclassified hypertension as $\geq 130/80$ mmHg (Stage 1: 130-139/80-89; Stage 2: $\geq 140/90$). Under current guidelines, both readings would already meet criteria for hypertension, changing the clinical reasoning in the question stem. Additionally, the framing of when to screen for secondary conditions or comorbidities may differ under current guidelines.

Q70

Current first-line treatments for postherpetic neuralgia per current guidelines include gabapentinoids (gabapentin, pregabalin) and tricyclic antidepressants as preferred first-line systemic agents, with topical lidocaine (5% patch) considered an option but not consistently ranked as the single most effective treatment. More importantly, choice D (herpes zoster vaccine) is now Shingrix (recombinant zoster vaccine, RZV), which has largely replaced Zostavax and has a different efficacy and safety profile—the question may reinforce outdated thinking about zoster vaccine options.

Q72

Pneumococcal vaccine recommendations have changed significantly. As of 2023 ACIP guidelines, PCV15 or PCV20 are now recommended for adults 65+, replacing the PCV13 (Pneumovax 13) + PPSV23 sequential strategy. PCV20 alone is now the preferred option for most adults, making PCV13 (Pneumovax 13) no longer the correct answer for routine use in a 65+ patient who previously received PPSV23.

Q74

Concussion management guidelines have evolved. Current evidence (and updated guidelines from 2023-2024) no longer recommends strict complete cognitive and physical rest; rather, early subsymptom-threshold aerobic activity is now recommended. Strict rest has been shown to potentially prolong recovery. The correct answer per current guidelines emphasizes individualized graded return to activity with early light aerobic exercise, not initial complete cognitive rest.

Q75

The correct answer listed is diltiazem (Cardizem), a non-dihydropyridine calcium channel blocker, which is actually contraindicated in HFrEF (EF 30%). Current ACC/AHA heart failure guidelines recommend ACE inhibitors (lisinopril), beta-blockers (carvedilol), and aldosterone antagonists (spironolactone) as evidence-based therapy for Stage C HFrEF. Diltiazem is harmful in this population. If the question intends diltiazem as the WRONG answer to avoid, the framing is confusing; but as listed with diltiazem as 'correct,' this is dangerously wrong and could mislead students.

Q81

The USPSTF updated its fall prevention recommendations. In 2018 and reaffirmed subsequently, the USPSTF recommended AGAINST vitamin D supplementation alone to prevent falls in community-dwelling adults 65 and older (Grade D recommendation). The current guidance no longer recommends vitamin D supplements for fall prevention in community-dwelling older adults, making the 2015 answer of Vitamin D actively misleading.

Q89

Blood pressure targets have been updated. The 2017 ACC/AHA guidelines lowered the definition of hypertension and targets. For older adults (≥ 65), current guidelines recommend a target of $<130/80$ mmHg for most patients. The answer of $<150/90$ mmHg (based on JNC 8 or older ACCF/AHA guidelines for elderly) is no longer considered the standard first-line target per current ACC/AHA 2017 guidelines, which a student today should follow.

Q103

CAP treatment guidelines have evolved. Current IDSA/ATS 2019 guidelines and subsequent updates recommend amoxicillin as preferred first-line monotherapy for outpatient CAP in otherwise healthy adults without comorbidities, with azithromycin as an alternative only in low macrolide-resistance areas. Given rising macrolide resistance, azithromycin monotherapy is no longer unambiguously first-line; amoxicillin is now preferred. A student selecting azithromycin as the best answer could be reinforcing outdated thinking.

Q108

The 2010 ACR fibromyalgia diagnostic criteria (revised 2011, reaffirmed in subsequent updates) replaced the tender point examination with the Widespread Pain Index (WPI) and Symptom Severity Scale (SSS). While 'structured symptom history' is broadly consistent with current criteria, the question was written in context of the older criteria debate. More importantly, the 2016 revised ACR criteria further refined this. The framing of the question and answer choices may mislead students about the current diagnostic approach, which is explicitly symptom-based scoring tools rather than a general 'structured symptom history,' and the answer could be interpreted as correct for the wrong reasons relative to current criteria.

Q113

The cardiovascular safety of NSAIDs has been re-evaluated. Current guidelines and the 2022 ACR osteoarthritis guidelines no longer single out naproxen as clearly superior cardiovascularly; celecoxib has been shown in the PRECISION trial (2016) to be non-inferior to naproxen and ibuprofen for CV outcomes and may be preferred in high-CV-risk patients. Selecting naproxen as the safest NSAID for a hypertensive patient could mislead students about current evidence and guidelines.

Q114

The correct answer 'Immediate tonsillectomy' is outdated. Current standard of care for peritonsillar abscess (the clinical picture described) is needle aspiration or incision and drainage, not immediate tonsillectomy. Tonsillectomy is not the first-line treatment for peritonsillar abscess in current guidelines.

Q118

Current guidelines (Endocrine Society 2016 and subsequent updates) recommend screening for primary aldosteronism using the aldosterone-to-renin ratio (ARR), which requires both plasma aldosterone concentration (PAC) and plasma renin activity (PRA) reported together as a ratio — not just measuring them independently as the question implies. Additionally, the threshold for screening and the preferred initial test framing has been updated. The question framing could mislead students about the current diagnostic approach.

Q122

The FDA now regulates e-cigarettes. The FDA finalized its 'deeming rule' in 2016, extending authority over e-cigarettes/vaping products. Answer C ('They are not regulated by the FDA') is now factually incorrect and would mislead students about current regulatory status.

Q125

Sepsis management guidelines (Surviving Sepsis Campaign) have been significantly updated since 2015. The aggressive 30 mL/kg fluid resuscitation mandate from the original Sepsis-3/SSC bundles has been revised. Subsequent trials (PROCESS, ARISE, ProMiSe, CLOVERS, CLASSIC) showed that aggressive fluid resuscitation is not always superior and may be harmful. Current guidelines (SSC 2021) recommend a more conservative, individualized approach to fluid resuscitation rather than universally 'aggressive' fluid resuscitation, making this answer potentially misleading.

Q127

PrEP guidelines have evolved significantly. Current CDC/USPSTF guidelines (2019, updated 2023) recommend HIV antigen/antibody combination immunoassay (4th generation testing) before starting PrEP, plus renal function testing (creatinine/eGFR) for TDF-based PrEP, and STI screening. Additionally, newer PrEP options exist (injectable cabotegravir, F/TAF). The question's framing and answer may be incomplete or misleading compared to current comprehensive PrEP initiation workup recommendations.

Q133

AAP updated vitamin D supplementation recommendations. Current AAP guidance (2022-2023) recommends 400 IU/day of vitamin D for breastfed infants starting shortly after birth, which is consistent with the answer, BUT iron supplementation recommendations have also been updated — AAP now recommends iron supplementation (1 mg/kg/day) for exclusively breastfed infants starting at 4 months of age, not just at later ages. At 2 months, iron supplementation is not yet recommended per current AAP guidelines, so the answer B remains correct; however, the question context could be confusing given updated iron guidance. Flagging as borderline since the iron answer choice A could mislead students about current AAP iron supplementation timing recommendations.

Q136

Testosterone replacement therapy monitoring has evolved. Current guidelines (Endocrine Society 2018, updated AUA/ISSM guidance) emphasize hematocrit monitoring remains relevant, but the 2023-2024 updated guidance also places greater emphasis on cardiovascular risk monitoring, and the FDA updated labeling around cardiovascular risks. Additionally, the TRAVERSE trial (2023) clarified cardiovascular safety, and current practice now includes more nuanced cardiovascular and PSA monitoring. The question is not wrong per se about hematocrit, but the monitoring landscape has changed enough that this question could mislead students about the full scope of required monitoring and relative priorities.

Q145

USPSTF HIV screening recommendation has been updated. Current USPSTF (2019) recommends screening for HIV in adolescents and adults aged 15 to 65, which matches answer B, BUT also recommends screening for younger adolescents and older adults at increased risk. More importantly, the current recommendation emphasizes screening all adults 15-65 regardless of risk, and the framing around 'high risk' has evolved. The answer B may still be technically correct but the question context and distractors could mislead students about nuances of current USPSTF guidance.

Q146

Management of gestational hypertension at term has evolved. ACOG and current guidelines (2022-2024) recommend delivery at 37 weeks for gestational hypertension, and the threshold for diagnosis and management has been updated. The correct answer (admit for induction) aligns with current practice at 39 weeks with BP 145/95, but the question context around preeclampsia diagnosis criteria and BP thresholds has changed with updated ACOG guidelines (e.g., severe features now defined at 160/110, and proteinuria is no longer required for preeclampsia diagnosis). Students could be misled about the diagnostic and management thresholds.

Q147

For obstructive sleep apnea with excessive daytime sleepiness, current first-line treatment is CPAP, not methylphenidate. Methylphenidate is not recommended as first-line treatment for OSA-related hypersomnia. The correct answer of methylphenidate (a stimulant) as the best next step is misleading; CPAP therapy or at minimum a sleep study/polysomnography referral would be current standard of care. Modafinil/armodafinil are the approved adjunct pharmacologic treatments for residual sleepiness in OSA, not methylphenidate.

Q149

Nitrofurantoin safety concerns have been updated. Current guidelines (FDA and ACOG) now include updated warnings about nitrofurantoin use in patients with renal insufficiency and in elderly patients. More critically for this question, if the cough and dyspnea are being attributed to nitrofurantoin pulmonary toxicity, the answer may still be correct, but nitrofurantoin's safety profile and contraindications (particularly in patients with eGFR <30-45) have been updated and could mislead students about current prescribing practice.

Q156

Current ACIP guidance (updated 2023-2024) states that people with egg allergy of any severity can receive any licensed, recommended, age-appropriate influenza vaccine in any setting. The recombinant vaccine is no longer specifically required or preferred for severe egg allergy. Additionally, influenza vaccines are now predominantly quadrivalent (trivalent formulations have been largely phased out), making answer choices referencing trivalent-only options potentially confusing and outdated.

Q164

Statin therapy recommendations have evolved. The 2013 ACC/AHA guidelines (which this question likely references) recommended moderate-intensity statins for diabetics 40-75 without high CV risk. However, the 2018 ACC/AHA cholesterol guidelines updated the approach for diabetics, incorporating risk calculators and risk-enhancing factors more explicitly. For a 55-year-old diabetic with no other risk factors, the 10-year ASCVD risk calculation is now central to the decision, and current guidelines (2018 ACC/AHA) may support moderate-intensity statin but the framing and decision pathway has changed enough that this question could mislead students about current methodology. Additionally, ADA 2024 guidelines recommend statin therapy for all diabetics aged 40-75 regardless of baseline LDL, making 'moderate-intensity' still reasonable but the clinical reasoning pathway taught here is outdated.

Q169

USPSTF hepatitis C screening recommendations have changed significantly. The 2020 USPSTF update expanded HCV screening to all adults aged 18-79 years regardless of risk factors, replacing the prior birth cohort (1945-1965) and risk-based screening approach. A 69-year-old male who has never smoked and takes only tamsulosin would now be screened for HCV simply based on age (he is within 18-79), not as a special screening target. The question implies HCV screening is a notable/special finding for this patient, which reflects the older birth cohort screening paradigm that has since been replaced by universal adult screening.

Q174

While adenosine remains appropriate for stable narrow-complex SVT, the question describes hemodynamic instability (BP 84/54) with a regular tachycardia. Current ACLS guidelines clearly indicate that unstable tachycardia with hemodynamic compromise should be treated with immediate synchronized electrical cardioversion, not adenosine. The correct answer per current guidelines should be electrical cardioversion (E), not adenosine (A).

Q177

Perioperative beta-blocker initiation guidelines have changed significantly. ACC/AHA guidelines now discourage initiating beta-blockers de novo immediately before non-cardiac surgery (within 24 hours) due to increased risk of stroke and death (POISE trial data). The question implies starting a beta-blocker the day before surgery is appropriate. Statins remain appropriate perioperatively, but the framing around beta-blockers in perioperative management has shifted enough that this question could reinforce outdated preoperative beta-blocker prescribing practices.

Q184

Colonoscopy surveillance intervals for hyperplastic polyps have been updated. Per the 2020 US Multi-Society Task Force guidelines, small (<10mm) hyperplastic polyps in the rectum or sigmoid colon are considered low-risk and follow-up is recommended at 10 years, which matches the answer. However, the classification and management of serrated polyps has been substantially revised — what were previously called 'hyperplastic polyps' may now be reclassified as sessile serrated lesions/adenomas under current WHO/pathology nomenclature, which carry different surveillance intervals (3-5 years). This question could reinforce outdated polyp terminology and incorrect surveillance intervals if lesions are actually sessile serrated adenomas.

Q185

Current USPSTF (2020) recommends screening for hepatitis B in adolescents and adults at increased risk, but the question framing about 'all newborns' not being recommended for screening is potentially misleading. More critically, universal hepatitis B vaccination of newborns is standard, and HBsAg screening of pregnant women (not 'all newborns') is the recommended approach. The question conflates screening with vaccination policy and could confuse students about current universal newborn HBV vaccination vs. screening recommendations.

Q189

While metformin remains first-line for type 2 diabetes, current ADA 2024 guidelines now recommend that patients with established ASCVD, heart failure, or CKD should receive an SGLT2 inhibitor or GLP-1 receptor agonist regardless of A1c or metformin status. The question context (A1c 8.0%, no stated comorbidities) still supports metformin, but the framing does not reflect current nuanced first-line decision-making that incorporates cardiovascular/renal risk as primary drivers of drug selection, potentially teaching students an oversimplified algorithm.

Q191

Current AAP/AAFP guidelines (2019 update) recommend behavior therapy alone as first-line for preschool children (ages 4-5) with ADHD, which aligns with the answer, BUT the question context mentions a 5-year-old and stimulant prescribing age thresholds have been clarified. More importantly, the 2019 AAP ADHD guideline expanded diagnosis/treatment to children 4-18 and updated first-line recommendations — a student should verify current age-specific guidance to avoid confusion with the nuances of the updated guideline.

Q193

Current CDC 2021 STI guidelines recommend treponemal tests (FTA-ABS or TPPA) for confirming suspected primary syphilis when the initial nontreponemal RPR is negative, because treponemal tests become positive earlier. The answer 'repeat RPR in 2 weeks' may no longer reflect best current practice; current guidelines favor performing a treponemal-specific test (FTA-ABS or TPPA) immediately when primary syphilis is clinically suspected and RPR is negative.

Q196

AAP updated bronchiolitis guidelines emphasize that supplemental oxygen is recommended when SpO₂ falls below 90% (not 92% as previously used). A patient with SpO₂ of 92% would not necessarily receive supplemental oxygen under current AAP 2014/updated guidance, and this threshold question could mislead students about current oxygen saturation targets for bronchiolitis management.

Q199

Current CDC Medical Eligibility Criteria and manufacturer labeling for the levonorgestrel IUD (Mirena) have been updated: if inserted within the first 7 days of the menstrual cycle, no backup contraception is needed. The answer 'no backup needed' is consistent, but the question framing about 'last day of menses' (day 5) aligns with current guidance. However, the 2023 updated Mirena labeling changed the approved duration from 5 years to 8 years, and newer LNG-IUD guidance includes updated insertion timing recommendations that could affect how this question is interpreted.

Q216

Current ACOG/ACIP guidelines (2023-2024) recommend both influenza AND Tdap during every pregnancy (Tdap ideally 27-36 weeks). Additionally, COVID-19 vaccine is now recommended in pregnancy. The question asking only about influenza as 'the one indicated for all pregnant women' is incomplete and could mislead students about current prenatal vaccine recommendations including Tdap and RSV vaccine (Abrysvo, recommended 32-36 weeks gestation as of 2023).

Q217

Current first-line treatment for nausea and vomiting of pregnancy per ACOG (2018, reaffirmed 2023) is the combination of doxylamine + pyridoxine (Diclegis/Bonjesta), not pyridoxine alone. Pyridoxine alone is still used but the preferred/guideline-recommended first-line pharmacologic option is the doxylamine-pyridoxine combination. A student today might be marked wrong for not recognizing the combination as preferred first-line over pyridoxine monotherapy.

Q219

The question appears to present a scenario where a statin (atorvastatin) is an option but fenofibrate is chosen, likely for isolated hypertriglyceridemia. However, without the full lipid values visible in the truncated question, the correct answer based on current ACC/AHA 2018 cholesterol guidelines may differ significantly. Current guidelines emphasize statins as first-line for cardiovascular risk reduction, and the role of fenofibrate and niacin has been substantially downgraded. Niacin (option A) is effectively no longer recommended for cardiovascular benefit per post-2013 trial data (AIM-HIGH, HPS2-THRIVE). This question could reinforce outdated lipid management thinking depending on the clinical scenario.

Q224

Carboprost is contraindicated in asthma. The question asks for the best uterotonic in a patient with asthma and the correct answer listed is carboprost (Hemabate), which is absolutely contraindicated in asthma. Current guidelines consistently list carboprost as contraindicated in asthma; misoprostol or oxytocin would be preferred. This answer could teach students a dangerous, incorrect principle.

Q226

Management of febrile neonates has evolved. Current guidelines (AAP 2021) for well-appearing febrile infants 22-28 days old allow for risk stratification using validated tools (e.g., Rochester, Step-by-Step, or Philadelphia criteria) rather than universal admission with full sepsis workup. The blanket 'admit all and start IV antibiotics' approach for a well-appearing 3-week-old may no longer reflect current best practice depending on risk stratification.

Q228

While adequate pain control is a reasonable component of delirium management, current guidelines (AGS, NICE) emphasize multicomponent non-pharmacological delirium prevention/management. The framing and choices include paroxetine (a sedating SSRI not recommended for acute agitation in delirium) and lorazepam (benzodiazepines are now strongly discouraged in non-alcohol-withdrawal delirium per current guidelines). The question may reinforce outdated thinking about delirium management options.

Q235

AOM treatment guidelines have been updated. For a 3-year-old with unilateral AOM without severe symptoms and no fever, current AAP 2022 guidelines support watchful waiting as an option before initiating antibiotics. The question presupposes antibiotic treatment is indicated and asks which regimen, which may not reflect current first-line approach of observation for mild cases in children ≥ 2 years.

Q236

For exercise-induced bronchoconstriction (EIB) without persistent asthma, current GINA and AAFA guidelines recommend pre-exercise short-acting beta-agonist (albuterol) as the preferred first-line treatment. Daily low-dose ICS is indicated for persistent asthma with EIB, not isolated EIB. The correct answer listed (daily ICS) may not be appropriate for isolated EIB, and albuterol pre-exercise is the current standard first-line recommendation for this specific indication.

Q240

Current ADA guidelines recommend that an abnormal urine albumin/creatinine ratio (≥ 30 mg/g) should be confirmed with a repeat test, which remains correct. However, current guidelines also recommend starting an ACE inhibitor or ARB for diabetic patients with microalbuminuria regardless of blood pressure, and SGLT2 inhibitors are now preferred agents with nephroprotective evidence. The question may also reflect outdated terminology ('microalbumin') and does not address whether ACEi/ARB initiation should be considered, potentially teaching incomplete management.

2016 Examination — 65 questions removed**Q2**

The USPSTF updated hepatitis C screening recommendations in 2020 to screen all adults aged 18-79, not just those born between 1945-1965. The birth cohort strategy is no longer the primary recommended approach, making answer B outdated and potentially misleading.

Q7

JNC 8 has been largely superseded. The 2017 ACC/AHA hypertension guidelines and subsequent updates recommend a BP target of $<130/80$ mmHg for most adults including older adults when tolerated. The 2023 ACC/AHA guidelines no longer endorse the $<150/90$ mmHg target for patients over 80. Teaching JNC 8 targets could mislead students on current board exams.

Q8

While azithromycin's QT-prolonging risk is real, current AHA/ACC guidance and FDA updates have evolved around this. More importantly, current IDSA/ATS community-acquired pneumonia guidelines (2019) have de-emphasized azithromycin monotherapy due to resistance concerns, and the clinical framing around azithromycin causing fatal arrhythmia has been nuanced in current literature. Additionally, azithromycin is still used but with updated risk context that could confuse students about current CAP treatment choices.

Q11

The question is truncated making the clinical scenario unclear, but if this is asking about initial workup for fatigue/anemia in a 16-year-old female on OCP with normal periods, hemoglobin electrophoresis as the best first test is questionable without knowing race/ethnicity context. More importantly, current guidelines (USPSTF, AAP, AAFP) emphasize ferritin as the preferred screening test for iron deficiency anemia in adolescent females, which is now recognized as the most common cause of anemia in this population. A student today might be confused that ferritin (choice B) is not the correct answer when current practice strongly supports iron deficiency evaluation first.

Q16

For a 55-year-old female with hypertension, obesity (BMI 34), and mildly elevated ALT with normal AST, the likely diagnosis is metabolic dysfunction-associated steatotic liver disease (MASLD), formerly called NAFLD/NASH. The question asks what to order next, and the correct answer is 'iron studies.' However, current guidelines (AGA, AASLD 2023) recommend non-invasive fibrosis assessment (FIB-4 score, liver elastography) as the preferred next step in evaluation of suspected MASLD, not iron studies. Additionally, the terminology NAFLD has been replaced by MASLD as of 2023, and the diagnostic and management pathway has been updated. A student today could be misled about the current recommended workup approach.

Q19

Colorectal cancer screening recommendations have changed significantly since 2016. The USPSTF updated its recommendation in 2021 to begin colorectal cancer screening at age 45 (lowered from 50), and the ACS made this change in 2018. The ACA marketplace coverage requirements for preventive services have also been subject to legal challenges (*Braidwood v. Becerra*), which has created uncertainty about mandatory coverage of USPSTF A/B recommendations. A student studying today could be confused about current coverage mandates and screening age thresholds.

Q21

Terminology 'detrusor instability' is outdated; current ICS/nosology uses 'overactive bladder' or 'detrusor overactivity.' A student learning this term may be confused on current board exams that use updated terminology.

Q22

With an A1c of 6.0% (below the 6.5% diagnostic threshold for diabetes) and only prediabetes/impaired fasting glucose, current ADA guidelines emphasize lifestyle intervention as primary treatment; pharmacotherapy choices and targets have evolved. Additionally, SGLT2 inhibitors (canagliflozin) and GLP-1 agonists are now preferred agents for obesity-related glucose management over acarbose. The 'correct' answer of acarbose does not reflect current preferred pharmacotherapy.

Q25

Concussion management guidelines have evolved. Current protocols (e.g., updated CDC HEADS UP, AAN, AAFP) no longer recommend strict 'complete' physical and cognitive rest; early sub-symptom-threshold aerobic activity is now recommended as it improves outcomes. The answer 'physical and cognitive rest' may teach outdated management.

Q26

USPSTF updated lung cancer screening criteria in 2021. Current recommendation is annual low-dose CT for adults aged 50-80 (lowered from 55) with a 20 pack-year history (lowered from 30) who currently smoke or quit within the past 15 years. This 70-year-old who quit 10 years ago with >30 pack-years still qualifies, but the question parameters could mislead students about current eligibility thresholds.

Q29

Current evidence and guidelines (AHA/ACC, CHEST) favor apixaban (Eliquis) over dabigatran for nonvalvular atrial fibrillation due to superior safety profile (lower bleeding risk, stroke reduction) demonstrated in ARISTOTLE trial. Multiple meta-analyses and current clinical practice guidelines rank apixaban as the preferred NOAC. Selecting dabigatran as correct could reinforce outdated prescribing preferences.

Q41

Current USPSTF and ASCCP guidelines (2018/2021) recommend that women aged 21-29 receive Pap test alone every 3 years (not cotesting, not reflex HPV). Reflex HPV testing is only triggered by an abnormal Pap result. The question asks what to do for a 25-year-old who hasn't had care in 5 years — the correct answer per current guidelines is a Pap test alone (every 3 years), not 'Pap test with reflex high-risk HPV testing' as a primary strategy. The framing of option E could mislead students into thinking reflex HPV is routinely ordered with the Pap for all 25-year-olds, which is not standard. Additionally, primary HPV testing alone (every 5 years) is now a preferred option for ages 25-65 per USPSTF 2018, which is not even listed as a choice.

Q46

The question describes macrocytosis in a patient taking naproxen (NSAID) and acetaminophen, with the correct answer listed as 'bone marrow suppression.' NSAIDs and acetaminophen are not well-established causes of macrocytosis/bone marrow suppression in an otherwise healthy patient. The clinical vignette likely intended to highlight folate deficiency or another cause. However, the answer 'bone marrow suppression' from OTC naproxen/acetaminophen reflects outdated or non-standard teaching and could mislead students about the actual mechanism and workup of macrocytosis. Current practice would prioritize B12/folate deficiency evaluation, alcohol use, or medications known to cause macrocytosis (e.g., methotrexate, hydroxyurea).

Q50

Current ACR and evidence-based guidelines for acute gout management have evolved. Colchicine is generally preferred as first-line for acute gout when renal function permits; however, in stage 4 CKD, colchicine dosing requires significant adjustment and is relatively contraindicated at standard doses. Prednisone is a reasonable answer for this patient given CKD stage 4 and recent surgery (where NSAIDs like indomethacin are contraindicated). This question is borderline but the reasoning around colchicine in CKD stage 4 has become more nuanced — low-dose colchicine (not standard dosing) may still be used cautiously in CKD stage 4 per current guidelines, making prednisone vs. colchicine a contested choice. Additionally, IL-1 inhibitors (e.g., anakinra) are now used in refractory cases. The question could mislead students about colchicine's role in CKD.

Q52

Current wound care guidelines for heel pressure ulcers with dry, stable eschar have evolved. The 2019 NPIAP (formerly NPUAP) guidelines indicate that stable, dry eschar on heels should NOT be debrided and should be kept dry rather than covered with a moist dressing. The correct answer of applying a moist dressing conflicts with current best practice for stable heel eschar, which recommends leaving it intact and dry.

Q57

Pneumococcal vaccine recommendations have changed significantly. The 13-valent PCV13 (Prevnar 13) is no longer routinely recommended for immunocompetent adults, and PCV15 or PCV20 are now the preferred pneumococcal conjugate vaccines per updated ACIP 2023 guidelines. While Tdap remains correct for cocoon strategy, the distractors involving pneumococcal vaccines could reinforce outdated vaccine options.

Q59

Current guidelines (IDSA, AAP, AAFP) for acute sinusitis favor watchful waiting without antibiotics for uncomplicated cases lasting less than 7-10 days without severe symptoms. A 'pocket prescription' approach has become more nuanced, and current guidelines more clearly recommend watchful waiting first without even providing a delayed prescription for patients meeting criteria for viral rhinosinusitis. This could mislead students about the preferred approach per current IDSA 2012/AAFP guidelines which emphasize no antibiotics rather than delayed prescriptions.

Q61

The question marks icatibant (a bradykinin receptor antagonist) as the correct answer for ACE inhibitor-induced angioedema. Current evidence and guidelines do not support icatibant as standard first-line therapy for ACE inhibitor-induced angioedema; its benefit in this setting has been questioned by randomized trials (e.g., CAMEO, FRESH trials) showing no significant benefit over placebo. Current management focuses on airway protection, stopping the ACE inhibitor, and supportive care. Teaching icatibant as the correct answer could mislead students about current best practice.

Q63

Barrett's esophagus surveillance intervals have been updated. Current ACG/AGA guidelines (2022-2023) recommend endoscopic surveillance every 3-5 years for non-dysplastic Barrett's esophagus, not a fixed every-3-years interval. The correct answer of 'every 3 years' may be too frequent per current guidelines, which allow up to every 5 years, and the framing could teach an outdated interval.

Q70

Current CDC/USPSTF guidelines strongly recommend offering pre-exposure prophylaxis (PrEP) to MSM at increased risk for HIV as a priority preventive intervention, and recommend HIV/STI screening every 3-6 months for sexually active MSM on PrEP. The question marks HBsAg testing as the correct answer while listing PrEP (choice A) and STI screening every 3 months (choice B) as distractors. For a 26-year-old MSM in 2024-2025, offering and discussing PrEP is a high-priority recommendation per USPSTF (Grade B, 2019) and CDC guidelines, and STI screening frequency recommendations have evolved. Selecting HBsAg testing over PrEP discussion as the best answer reflects outdated prioritization and could mislead students about current preventive care priorities for this population.

Q71

USPSTF and current guidelines on calcium/vitamin D supplementation have evolved. The USPSTF (2018) found insufficient evidence to recommend vitamin D supplementation for fracture prevention in community-dwelling adults, and updated guidance questions routine calcium+D supplementation for osteoporosis prevention. Additionally, the answer does not reflect current FRAX-guided management approach or updated NOF/ASBMR guidance. A student could be misled about calcium 1200mg+vitamin D 800 IU as the correct answer when current evidence and guidelines are more nuanced or even cautionary about supplementation.

Q75

HPV vaccine schedule and eligible age groups have changed significantly. Current ACIP recommendations (2019+) extend HPV vaccination recommendation through age 26 for all, and shared decision-making up to age 45. More importantly, the HPV vaccine series starting age and the number of doses at age 12 are still correct, but the question omits Tdap which is due at age 11-12 and is a standard catch-up vaccine. A 12-year-old who has been unvaccinated since age 6 would also need MMR booster, varicella booster, and Tdap per current catch-up schedules, making the answer incomplete/misleading. Also, varicella is missing from the correct answer but a child vaccinated at age 6 likely received only one varicella dose and may need a second.

Q77

Ranitidine (Zantac) was withdrawn from the US market by the FDA in April 2020 due to NDMA contamination concerns. Including it as an answer choice could reinforce prescribing of a withdrawn medication, and students should not be learning ranitidine as a treatment option.

Q78

Henoch-Schönlein purpura (HSP) has been renamed IgA vasculitis (IgAV) per current ACR/EULAR 2022 nomenclature and classification criteria. A student today should know this condition by its current name IgA vasculitis, and the older term could reinforce outdated nosology on current board exams.

Q83

JNC 8 guidelines referenced are no longer the current standard. The 2017 ACC/AHA hypertension guidelines replaced JNC 8 and changed BP thresholds and treatment recommendations. However, the answer (thiazide diuretic or CCB for African Americans) remains consistent with current guidelines. The question is flagged because it specifically references 'JNC 8 panel guidelines' which have been superseded, and students should learn current 2017 ACC/AHA framework. Additionally, the 2017 guidelines lowered the hypertension threshold to 130/80 mmHg, changing the overall context of hypertension management questions.

Q84

Current GINA 2022-2024 guidelines have updated exercise-induced bronchoconstriction (EIB) management. GINA now recommends against regular use of SABA monotherapy as the sole treatment even for EIB, as it can lead to tolerance and inadequate control. Current preferred approach for EIB may include as-needed low-dose ICS-formoterol or pre-exercise ICS-SABA, and the role of pre-exercise albuterol alone without controller therapy has been reconsidered. This could teach students an approach that is no longer fully aligned with current asthma guidelines.

Q88

This question references the 2013 ACC/AHA cholesterol guidelines framework. While moderate/high-intensity statin therapy remains correct, the 2018 ACC/AHA Guideline on Management of Blood Cholesterol updated the framework significantly, adding risk-enhancing factors, CAC scoring, and new thresholds for PCSK9 inhibitor use. The clinical context (45-year-old male smoker with hypertension) may now warrant different risk calculation and shared decision-making discussions per 2018 guidelines. The question could mislead students about how risk is calculated and when to initiate therapy.

Q92

Pneumococcal vaccine recommendations have changed significantly. The CDC/ACIP now recommends PCV15 or PCV20 for adults 65+. If PCV15 is used, PPSV23 follows; if PCV20 is used, no additional pneumococcal vaccine is needed. The old PCV13+PPSV23 sequential schedule is no longer the standard recommendation, and PCV13 is no longer routinely recommended for immunocompetent adults 65+. The correct answer 'PCV13 now and PPSV23 in 1 year' would be incorrect by current ACIP 2022+ guidelines.

Q93

Current guidelines (ACG, USMSTF, ACS) recommend first-degree relative with CRC diagnosed before age 60 should begin colonoscopy at age 40 OR 10 years before the relative's diagnosis, whichever is earlier. With a father diagnosed at 45, screening should start at 35 (10 years before) — so the answer of 35 may still be defensible. However, some current guidelines (ACG 2021) recommend age 40 or 10 years before the youngest affected relative's diagnosis. The ACG 2021 guideline recommends starting at age 40 for a first-degree relative diagnosed at any age, which would make answer C (40) correct rather than B (35). This discrepancy warrants flagging.

Q100

The question references the 2014 GOLD guideline specifically, which is outdated. Current GOLD 2023/2024 guidelines still recommend systemic corticosteroids for COPD exacerbations with prednisone 40 mg for 5 days, so the answer itself remains current. However, the question explicitly cites the '2014 GOLD guideline' which could confuse students into thinking they need to know an outdated guideline version. This is a borderline flag, but referencing a specific outdated guideline version is problematic.

Q101

While PTT can be prolonged in von Willebrand disease (especially type 3), current guidelines emphasize that vWD diagnosis requires specific vWF antigen, vWF activity (ristocetin cofactor), and factor VIII assays. PTT is frequently NORMAL in vWD (types 1 and 2), making this answer potentially misleading. A student learning that isolated prolonged PTT is the most consistent finding with vWD would be mistaught — most vWD patients have a normal PTT.

Q108

Ranitidine (an H2 blocker mentioned in choice D) was withdrawn from the US market in 2020 due to NDMA contamination concerns. Including it as a distractor in a current exam could cause confusion. Additionally, current osteoporosis guidelines (AAACE/ACE 2020, Endocrine Society) increasingly favor anabolic agents like teriparatide or romosozumab as first-line therapy for very high-risk fracture patients (e.g., hip fracture at age 82), making alendronate potentially no longer the unambiguous best answer for this high-risk patient.

Q109

ACIP guidelines on influenza vaccination in egg-allergic patients have been updated. Current ACIP recommendations (2023-2024) state that persons with egg allergy of any severity (including anaphylaxis) can receive any licensed, recommended, age-appropriate influenza vaccine without special precautions beyond those used for any vaccine recipient. The previous multi-step protocols or restrictions no longer apply. Additionally, live attenuated influenza vaccine (LAIV/FluMist) is now an option for eligible non-immunocompromised adults. The framing and correct answer choice may no longer reflect the most current simplified guidance.

Q115

The USPSTF updated its aspirin recommendations significantly. In 2022, USPSTF recommended against initiating aspirin use for primary prevention of CVD in adults 60 and older, and stated that the decision to initiate low-dose aspirin for primary prevention in adults aged 40-59 with $\geq 10\%$ 10-year CVD risk should be an individual decision. For a healthy 45-year-old with no stated CVD risk factors, aspirin for primary prevention is now generally not recommended. The question context and correct answer (discontinuing aspirin) may still be directionally correct but the framing could mislead students about current nuanced primary prevention guidance.

Q116

PID treatment guidelines have been updated. The 2021 CDC STI Treatment Guidelines changed the recommended ceftriaxone dose for outpatient PID from 250 mg IM to 500 mg IM. Additionally, current CDC guidelines updated inpatient regimens. The question's choice A lists 250 mg ceftriaxone IM, which is now outdated. If students review this question, they may be confused about whether hospitalization criteria or outpatient regimens have changed, and the 250 mg dose listed as a distractor is no longer the correct outpatient dose, potentially reinforcing an outdated regimen.

Q120

Since 2016, additional FDA-approved weight loss medications have become available (e.g., semaglutide/Wegovy, tirzepatide/Zepbound) that are now first-line pharmacologic options for obesity management and may be appropriate before or instead of bariatric surgery for some patients. The question framing and answer choices do not reflect current obesity pharmacotherapy options, which could mislead students about the current standard of care for severe obesity. GLP-1 receptor agonists are now considered major therapeutic options that can achieve substantial weight loss.

Q125

While IV metoclopramide is still used for migraine, the question context and answer choices include oral butalbital combinations and IV hydromorphone. Current guidelines (AHS, ACEP) strongly discourage opioids and butalbital-containing medications for migraine due to medication overuse headache risk and inferior outcomes. The framing of this question and its distractors may reinforce outdated thinking about acceptable migraine treatments. Additionally, supplemental oxygen as a distractor could confuse students who know it is first-line for cluster headache, not migraine. The correct answer (IV metoclopramide) remains reasonable but the question environment could mislead.

Q129

Nebulized hypertonic saline is no longer recommended as a standard treatment for bronchiolitis. Current AAP 2014/updated guidelines indicate that hypertonic saline should NOT be used in the emergency department setting and evidence for inpatient use is weak; major guideline updates and systematic reviews since 2016 have downgraded or removed this recommendation. The current answer for appropriate treatment in bronchiolitis is supportive care only, as albuterol, epinephrine, hypertonic saline, and corticosteroids are all not routinely recommended. This question could mislead students into thinking hypertonic saline is an appropriate treatment.

Q130

Current guidelines recommend late-night salivary cortisol or 1mg overnight dexamethasone suppression test as first-line screening for Cushing syndrome, not 24-hour urine free cortisol as the single best answer. The 24-hour urine free cortisol is one of three acceptable screening tests but is no longer considered the preferred first-line test per current Endocrine Society guidelines. A student today might be marked wrong for selecting this over late-night salivary cortisol on a current exam.

Q133

Evaluation of microscopic hematuria guidelines have been updated. The 2020 AUA Microhematuria Guidelines revised the evaluation pathway, changing thresholds and risk stratification. The 2020 guideline defines clinically significant microscopic hematuria as ≥ 3 RBC/hpf and introduced a risk-stratified approach. BUN/creatinine plus CT urography plus cystoscopy may not be the universal first-line workup for all patients under current risk-stratified guidelines, and the older approach of universal upper and lower tract evaluation has been refined.

Q136

CAP treatment guidelines have been updated. The 2019 ATS/IDSA CAP guidelines and subsequent updates changed preferred regimens. For hospitalized non-ICU patients, the combination of a beta-lactam plus macrolide remains acceptable, but azithromycin use in elderly patients with cardiovascular risk and QT concerns has been scrutinized. More importantly, current IDSA/ATS 2019 guidelines de-emphasize azithromycin monotherapy and have updated severity scoring. The answer may still be defensible but the specific regimen framing from 2016 may not align perfectly with current 2019 ATS/IDSA guidance which also incorporates PSI/PORT score more explicitly. Flag as borderline due to updated guideline framing.

Q142

Current CDC/IDSA guidelines for traveler's diarrhea to South Asia (India) now recommend azithromycin as preferred, but rifaximin has become a major recommended option, and importantly fluoroquinolone resistance in South Asia is high making ciprofloxacin less preferred. However, the bigger issue is that current 2017+ CDC guidelines updated recommendations — rifaximin is now a first-line option for non-dysenteric TD, and the question may mislead students about the current landscape. Additionally, current guidelines (CDC Yellow Book 2024) list azithromycin as preferred for South Asia specifically due to fluoroquinolone-resistant *Campylobacter*, which remains correct, but the exclusion of rifaximin as a correct answer and the framing could mislead students about current multi-drug options.

Q147

Current ADA and geriatric guidelines (2023-2024) have evolved on glycemic targets in elderly patients with dementia. Current ADA standards suggest an A1c target of 8-9% is reasonable for patients with limited life expectancy, multiple comorbidities, or advanced dementia, which still aligns broadly, but the blood pressure target answer choice of <130 mmHg now reflects current ACC/AHA 2017 guidelines as a standard target even in elderly, and the framing around 'not appropriate' for BP <130 could mislead students. More critically, current guidelines on LDL targets and statin use in frail elderly have also evolved — the 2023 ACC/AHA and AGS Beers Criteria have nuanced recommendations about deprescribing statins in limited life expectancy patients, making the LDL distractor potentially confusing. The A1c 8-9% answer remains consistent with current guidelines for this population.

Q154

Guidelines for uncomplicated skin abscesses have evolved. Current IDSA and evidence from trials (NEJM 2017) support adjunctive antibiotics (TMP-SMX or clindamycin) after I&D for abscesses ≥ 2 cm or in certain clinical contexts. The 2017 IDSA SSTI guidelines recommend considering TMP-SMX or clindamycin after I&D even for uncomplicated abscesses, and multiple RCTs showed improved cure rates with antibiotics. 'Observation only' is no longer the clearly preferred answer for a 2cm abscess.

Q155

Current fibromyalgia management guidelines (ACR, EULAR 2023) emphasize multimodal therapy with exercise/aerobic activity as the single most effective intervention. FDA-approved agents (duloxetine, milnacipran, pregabalin) are considered first-line pharmacotherapy. TCAs are considered second-line; designating TCAs as 'most effective' is inconsistent with current evidence and guidelines.

Q157

AAP 2010/reaffirmed guidance recommends iron supplementation for exclusively breastfed infants starting at 4 months, but the AAP 2022 updated nutrition guidelines and USPSTF recommendations should be verified. While 4 months remains a common answer, some current sources cite introduction of iron-rich complementary foods at 6 months as sufficient. This question warrants flagging given evolving AAP guidance on infant iron supplementation timing.

Q163

USPSTF recommendations have been updated since 2016. For colorectal cancer screening, the USPSTF now recommends screening starting at age 45 (updated 2021), which changes the calculus on answer D (70-year-old male colonoscopy is still recommended). More critically, the question asks which has a D recommendation — the Pap smear after hysterectomy for benign disease remains a D recommendation and the answer C is still correct, BUT answer D (colonoscopy for 70-year-old) is now a B recommendation per 2021 USPSTF update (ages 45-75), so a student might be confused about whether D could be the answer. The question is borderline but could mislead students about current colorectal cancer screening age thresholds.

Q165

The diagnosis of 'somatization disorder' is outdated terminology. The DSM-5 (2013) replaced somatization disorder with 'somatic symptom disorder,' which uses different diagnostic criteria. A student studying today would encounter the new nosology and this question reinforces obsolete terminology.

Q177

USPSTF screening recommendations for adolescents have been updated. The correct answer 'Depression' remains valid (USPSTF B recommendation for screening adolescents 12-18), but 'Chlamydia infection' is also a USPSTF B recommendation for all sexually active females ≤ 24 years including adolescents, making this question potentially misleading. Additionally, 'Alcohol use' now has a USPSTF B recommendation for screening in adolescents 18 and older as of updated guidance, and the question could teach students that chlamydia screening in adolescent females is NOT recommended when it actually is.

Q184

While vaginal estradiol remains appropriate, the terminology and product landscape have changed. More importantly, current NAMS/ACOG guidelines now also recommend ospemifene and vaginal DHEA (prasterone) as first-line options for genitourinary syndrome of menopause (GSM) — a term that has replaced 'vaginal atrophy/dryness.' The question uses older framing and omits newer first-line alternatives that a student today should know, potentially reinforcing an incomplete picture of current GSM management.

Q186

Current guidelines (ACC/AHA) recommend dual antiplatelet therapy with clopidogrel plus low-dose aspirin (81 mg) after drug-eluting stent placement, which remains correct, BUT the correct answer here labels clopidogrel as the P2Y12 inhibitor of choice without acknowledging that ticagrelor is now the preferred P2Y12 inhibitor over clopidogrel for ACS per current ACC/AHA guidelines. A student today could be misled into thinking clopidogrel is the preferred agent when ticagrelor is generally preferred for ACS with stenting. The question framing reinforces clopidogrel-first thinking that is no longer current best practice.

Q188

The answer lists intravenous insulin and glucose as best initial management for severe hyperkalemia (6.8 mEq/L with cardiac risk), but current guidelines emphasize that intravenous calcium gluconate should be given FIRST to stabilize cardiac membranes when potassium is this severely elevated (especially in a symptomatic or high-risk patient), before insulin/glucose. Choosing insulin/glucose over calcium gluconate as the 'best initial' step could be misleading and potentially dangerous in current practice. Additionally, sodium polystyrene sulfonate (Kayexalate) has fallen out of favor and newer agents (patiromer, sodium zirconium cyclosilicate) are now preferred for ongoing potassium management.

Q194

Anticoagulation duration guidelines have been updated. Current CHEST/ASH guidelines (2020-2021) still support 3 months for provoked PE from surgery, which remains correct. However, DOACs are now strongly preferred over traditional anticoagulants as first-line therapy, and the question does not address agent selection. While the duration answer remains accurate, the question context is outdated enough that it could reinforce incomplete thinking about modern anticoagulation management.

Q195

SIDS risk factors have been updated. 'Apparent life-threatening event' (ALTE) terminology has been replaced by 'Brief Resolved Unexplained Event' (BRUE) by AAP in 2016, and the relationship between BRUE/ALTE and SIDS has been clarified. Additionally, AAP safe sleep guidelines have been significantly updated through 2022, and the risk factor profile has been refined. Side sleeping is indeed a risk factor, but the question references outdated terminology (ALTE) and outdated ethnicity data that could confuse students.

Q198

Management of gestational hypertension has evolved. Current ACOG guidelines (updated through 2022-2023) recommend delivery at 37 weeks for gestational hypertension without severe features, which aligns with answer B, but the framing and thresholds for what constitutes gestational hypertension vs preeclampsia without severe features have been updated. The clinical scenario describes a patient who may meet criteria differently under current ACOG definitions, and management pathways have been updated making this question potentially misleading.

Q206

Current IDSA/AAP guidelines recommend mupirocin or retapamulin topical therapy as first-line for localized nonbullous impetigo. Oral cephalexin remains an option for extensive disease, but topical antibiotics are now preferred first-line for limited nonbullous impetigo. The question may reinforce oral antibiotic use when topical therapy is now guideline-preferred.

Q207

Current evidence and guidelines (including AAP and multiple systematic reviews) support ondansetron as the preferred antiemetic for gastroenteritis, particularly in pediatric and adult outpatient settings. Metoclopramide is not considered first-line due to extrapyramidal side effects and FDA black box warning for tardive dyskinesia. Flagging because the framing may mislead students about which agent is actually preferred in practice today, even though the mechanism fact about metoclopramide is correct.

Q215

USPSTF fluoride supplementation recommendation has been updated. Current USPSTF guidance (2021) recommends prescribing oral fluoride supplementation starting at age 6 months for children whose water supply is deficient in fluoride, but the question's answer choices and framing may not reflect the most current nuanced guidance distinguishing supplementation from fluoride varnish application (which is recommended for all children once primary teeth erupt). The 2021 USPSTF update recommends fluoride varnish application by primary care clinicians for all children starting when primary teeth erupt, which changes the clinical context of the question.

Q216

Palivizumab (Synagis) RSV prophylaxis eligibility criteria have been updated by the AAP (2014, 2022, and 2023 updates). Current AAP guidance significantly restricts palivizumab use even in preterm infants. Additionally, the FDA approved nirsevimab (Beyfortus) in 2023 as a long-acting RSV monoclonal antibody now preferred over palivizumab for RSV prophylaxis in infants, fundamentally changing the answer to this type of question. A student learning palivizumab as the go-to answer would be taught outdated practice.

Q217

Management of WPW with atrial fibrillation (which this EKG pattern with wide, irregular, fast rhythm likely depicts) has important nuances. Verapamil is actually CONTRAINDICATED in WPW with pre-excitation/atrial fibrillation as it can accelerate conduction through the accessory pathway and cause ventricular fibrillation. If the EKG shows a delta wave or pre-excitation pattern (likely given the clinical context of a young patient with recurrent palpitations), recommending verapamil as the correct answer could teach students a dangerous and incorrect approach. Current guidelines recommend procainamide or DC cardioversion for WPW with AF.

Q221

Current ACOG and ACAAI guidelines recommend vitamin B6 (pyridoxine) alone OR vitamin B6 plus doxylamine as first-line therapy for nausea and vomiting of pregnancy. The answer 'Vitamin B' is ambiguous and incomplete — the specific recommendation is Vitamin B6 (pyridoxine) 10-25mg TID, often combined with doxylamine (Diclegis/Bonjesta), which is the FDA-approved first-line regimen. Ginger is also recognized as a first-line non-pharmacologic option by multiple current guidelines (ACOG 2018, 2024 updates), potentially making this question misleading as both D and A could be considered first-line depending on the guideline consulted.

Q226

BP classification and management thresholds have changed. Per 2017 ACC/AHA guidelines (still current), 142/92 mmHg is now classified as Stage 2 hypertension (not 'mild'), and lifestyle modification alone may be insufficient — pharmacologic therapy is recommended for Stage 2 HTN. Additionally, the 2017 ACC/AHA guidelines lowered the HTN threshold to $\geq 130/80$ mmHg. A student learning from this question may be misled about the current classification and appropriate management intensity for this blood pressure level.

Q228

AHA/ACC and sports medicine guidelines on preparticipation cardiovascular screening have been updated and debated since 2016. The 2017 AHA/ACC scientific statement updated recommendations, and subsequent AAFP and AAP guidance has evolved. The current AAFP recommendation against routine EKG screening remains, but the nuances and supporting evidence base have been updated. This question may still reflect current practice but should be verified against the most recent AAFP/AHA joint statements from 2022-2024, as international guidelines (ESC) recommend EKG and the debate is ongoing — a student could be misled about the strength and basis of the recommendation.

Q232

USPSTF AAA screening recommendation specifies eligibility as men aged 65-75 who have EVER smoked (at least 100 cigarettes lifetime), not based on quitting status. This 70-year-old who quit 10 years ago still qualifies. However, the screening modality answer (one-time abdominal duplex ultrasonography) may be confused with 'abdominal ultrasound' — the USPSTF recommends one-time abdominal aortic ultrasound, not specifically 'duplex ultrasonography.' More importantly, the question stem says he quit 10 years ago but smoked 'for many years' — the current USPSTF criteria require 100+ cigarette lifetime history which this patient likely meets, but the framing could mislead students about current eligibility criteria. The correct modality per current guidelines is abdominal ultrasound (not specifically duplex), which could teach incorrect terminology.

Q239

Current WHO Medical Eligibility Criteria (MEC) and ACOG guidelines classify smoking ≥ 15 cigarettes/day in women ≥ 35 as a Category 4 (absolute contraindication) for combined oral contraceptives. However, sickle cell disease is classified as Category 2 (advantages outweigh risks) — not a contraindication — which is correct. But the question asks about a 42-year-old smoking 1 pack/day (20 cigarettes), which remains an absolute contraindication. The answer E is still correct per current guidelines. However, obesity (BMI >30) in a 42-year-old is Category 2, not a contraindication, which is also consistent with current guidance. This question appears consistent with current practice and should be carefully reviewed — the answer remains correct, but the framing of absolute vs. relative contraindications per current MEC 2024 is worth verifying. On balance, this appears still current.

2017 Examination — 60 questions removed

Q8

The question asks which hypoglycemic medication is 'proven to reduce mortality in type 2 diabetes.' Since 2017, multiple agents have demonstrated cardiovascular mortality benefit in large outcomes trials: SGLT2 inhibitors (empagliflozin, canagliflozin, dapagliflozin) and GLP-1 receptor agonists (liraglutide, semaglutide) have robust evidence for CV mortality reduction. Metformin's mortality benefit (based largely on UKPDS subgroup data) is now considered weaker than these newer agents. A student today could be misled into thinking metformin is the only or primary mortality-reducing agent in T2DM.

Q10

USPSTF and CDC hepatitis C screening recommendations have changed significantly. The 2020 USPSTF recommendation expanded hepatitis C screening to all adults aged 18-79 years (Grade B), regardless of risk factors. For this patient (38-year-old with multiple sexual partners), hepatitis C antibody screening would now be recommended as routine screening, making answer B also correct and potentially more current than the hepatitis B focus of answer A. The question framing around what 'should' be screened may now yield a different best answer.

Q14

For BPH with moderate symptoms and normal prostate size, alpha-blockers (tamsulosin, silodosin) are first-line per current AUA guidelines. Finasteride is indicated for enlarged prostates (typically PSA >1.5 or prostate volume >30cc). This patient has a PSA of 0.75 and normal exam, suggesting a small gland where finasteride would not be first-line. Marking finasteride as correct could mislead students about current BPH management.

Q16

DAPT duration guidelines after drug-eluting stent placement have been updated. Per 2022 ACC/AHA guidelines, 6 months of DAPT is now considered appropriate for many stable patients with drug-eluting stents, with shorter durations (1-3 months) acceptable in high bleeding risk patients. 'At least 12 months' as a blanket answer for ACS with DES may no longer reflect the nuanced current recommendations, though 12 months remains reasonable for ACS; the framing of this as the single correct answer may be misleading given evolving guideline updates.

Q19

Current ACIP guidelines (updated 2023-2024) state that egg allergy of any severity is no longer a contraindication to influenza vaccination and no special precautions are needed beyond those recommended for any vaccine recipient. The question premise treats egg allergy as a relevant consideration requiring the answer to address it, which could reinforce outdated thinking. Additionally, current guidelines recommend the recombinant or cell-based flu vaccine as preferred alternatives for egg-allergic patients, not simply 'standard inactivated seasonal vaccine,' depending on the severity of prior reaction.

Q23

Quetiapine as the correct answer for augmentation/treatment of persistent depression is questionable in current practice context. More importantly, current guidelines (APA 2023, CANMAT) emphasize atypical antipsychotics as augmentation only after adequate SSRI/SNRI trials, and the framing of quetiapine as the preferred agent over SNRIs or other augmentation strategies does not align well with current first-line augmentation recommendations which now also include newer agents. Additionally, atypical antipsychotics carry significant metabolic risk warnings that have been updated.

Q26

Management of febrile infants has been significantly updated. A 40-day-old (>28 days but ≤60 days) with fever requires a full sepsis workup per current AAP 2021 guidelines, not home care with reevaluation in 24 hours. The correct answer 'B' contradicts current practice which mandates blood culture, urinalysis/urine culture, and CSF studies for infants in this age group with fever.

Q27

While cilostazol remains a treatment option for claudication, current AHA/ACC 2024 peripheral artery disease guidelines place greater emphasis on supervised exercise therapy as first-line treatment before pharmacotherapy. The question as framed may mislead students into thinking cilostazol is the primary intervention rather than structured exercise rehabilitation, which is now the cornerstone of PAD claudication management.

Q31

Current ADA and AAEM guidelines (2022-2024) now recommend SNRIs (duloxetine) or the alpha-2-delta ligands (pregabalin, gabapentin) as first-line, but importantly the 2022 ADA Standards of Care list duloxetine, gabapentin, pregabalin, and tricyclics as options with duloxetine often cited first. More critically, the 2023 ADA guidelines and other current sources list SNRIs (duloxetine) as preferred first-line over pregabalin for diabetic peripheral neuropathy pain, which could mislead students studying current practice. Pregabalin is still acceptable but may no longer be the single best first-line answer on current exams.

Q37

Management of hypertension in pregnancy has been updated. Current ACOG guidelines (updated 2022-2024) recommend antihypertensive treatment initiation at BP ≥140/90 mmHg in pregnancy (threshold lowered from 160/110), and timing/mode of delivery for preeclampsia has been refined. At 35 weeks with preeclampsia without severe features, current ACOG guidance recommends delivery at 37 weeks (not immediate induction), making 'immediate induction of labor' at 35 weeks potentially incorrect or at least reflecting outdated thresholds. This question could teach incorrect management.

Q38

The FDA added and later revised the black box warning for varenicline (Chantix). Additionally, varenicline (brand name Chantix) was recalled/discontinued in the US market in 2021 due to nitrosamine contamination, though generic versions returned. More importantly, current guidelines (USPSTF 2021, AAFP) now recommend combination NRT or varenicline as first-line with roughly equivalent standing, and the question context may be affected by the market withdrawal history. Students should know the current availability and guideline status of varenicline generics.

Q49

AHA updated dental prophylaxis guidelines: clindamycin is no longer recommended for penicillin-allergic patients due to risk of *C. difficile* colitis. Current AHA/ADA guidance (2021 update) recommends azithromycin or clarithromycin (or cephalexin if not allergic to cephalosporins) instead of clindamycin for penicillin-allergic patients requiring IE prophylaxis before dental procedures.

Q54

Prazosin for PTSD nightmares is no longer recommended as first-line. A 2018 VA/DoD and subsequent AAFP-aligned guidance showed prazosin was not superior to placebo for PTSD nightmares in a large RCT (NEJM 2018), and current VA/DoD and APA guidelines no longer recommend prazosin for PTSD-related sleep disturbance. Teaching this as correct could mislead students on current boards.

Q57

Current guidelines (ACG 2024, AGA) for severe acute pancreatitis now favor early oral feeding when tolerated over routine nasogastric enteral nutrition. When enteral nutrition is needed, nasojejunal and nasogastric routes are considered equivalent, and early oral feeding is preferred over continuous nasogastric tube feeding in many patients. The framing of continuous nasogastric enteral nutrition as the best answer may no longer reflect current best practice.

Q59

CKD staging and eGFR thresholds have been updated. The 2021 KDIGO guidelines revised eGFR equations (CKD-EPI 2021 race-free equation), which changes eGFR values and potentially CKD staging for the same creatinine. Additionally, the hemoglobin threshold and management targets for CKD-related anemia have been updated with new ESA and HIF-PHI guidance. The question's clinical framing around an eGFR of 36 and what is most concerning may yield a different answer under current CKD staging and anemia management guidelines.

Q63

USPSTF depression screening recommendations have been updated. The 2023 USPSTF recommendation on screening for depression in adults remains, but the 2023 update on adolescents (ages 12-18) also recommends screening. More importantly, the 2023 USPSTF draft/final guidance on anxiety disorder screening now recommends screening adults under 65 for anxiety disorders, which could make answer B also correct on a current exam, making this question ambiguous and potentially misleading.

Q68

AAP dyslipidemia screening recommendations have been debated and updated. The USPSTF in 2023 concluded insufficient evidence for lipid screening in children and adolescents, which conflicts with the AAP recommendation cited. A student could be confused about which guideline applies, and current USPSTF (which ABFM tends to follow) does not recommend universal dyslipidemia screening at age 11, making this potentially misleading.

Q70

Current palliative care and hospice guidelines have evolved regarding management of terminal delirium. While determining underlying cause is reasonable, current evidence-based guidelines (AAHPM, NHPKO) increasingly recognize that in actively dying patients with refractory terminal delirium, symptomatic treatment (including low-dose antipsychotics or benzodiazepines) may be appropriate first-line alongside or instead of exhaustive workup. The framing of the question could teach students to pursue aggressive workup in an actively dying patient, which conflicts with current palliative care best practices.

Q76

The question presents a classic cystic fibrosis clinical picture (recurrent lower-lobe bronchopneumonia, failure to thrive, sibling death) but marks the correct answer as gastroesophageal reflux. This is misleading and would teach students incorrect diagnostic reasoning. Current guidelines and clinical teaching would point to cystic fibrosis as the primary diagnosis to pursue in this scenario. Students studying today could be misled into deprioritizing CF workup.

Q78

Treatment of acute chest syndrome in sickle cell disease has evolved. Current NHLBI and ASH guidelines emphasize empiric broad-spectrum antibiotics covering both atypical and typical organisms (a beta-lactam PLUS a macrolide such as azithromycin), along with incentive spirometry, simple or exchange transfusion, and bronchodilators. Listing azithromycin alone as the correct answer without context of combination therapy could reinforce incomplete management and mislead students about current comprehensive treatment protocols.

Q82

Current IDSA/AUA guidelines still list TMP-SMX as a first-line option for uncomplicated UTI, but only when local *E. coli* resistance is <20%. More importantly, fluoroquinolone stewardship guidance and updated guidelines now emphasize nitrofurantoin and fosfomycin as preferred first-line agents over TMP-SMX to preserve broader-spectrum antibiotics. A student today might be taught nitrofurantoin is the preferred first-line agent, making TMP-SMX a second-line choice. This question could reinforce outdated prescribing hierarchy.

Q86

Current ACC/AHA stable ischemic heart disease guidelines recommend a beta-blocker (e.g., metoprolol succinate) as the first-line antianginal agent for stable angina, not isosorbide mononitrate. Long-acting nitrates are second-line or adjunctive therapy. The correct answer of isosorbide mononitrate over metoprolol succinate contradicts current guideline-recommended first-line therapy and could mislead students into deprioritizing beta-blockers.

Q90

Guidelines for cancer-associated VTE have changed significantly. The 2023 ASCO, NCCN, and ASH guidelines now recommend direct oral anticoagulants (specifically apixaban or rivaroxaban) as preferred first-line options over LMWH (enoxaparin) for cancer-associated DVT in patients without high bleeding risk GI or GU malignancies. Enoxaparin was the standard of care at the time of this question but has been largely supplanted by DOACs in this setting. A student today selecting enoxaparin over apixaban would be learning outdated practice.

Q94

Use of atypical antipsychotics (including aripiprazole) for behavioral disturbances in dementia carries an FDA black box warning for increased mortality. Current guidelines (APA 2023, AGS) emphasize that antipsychotics should be used only when nonpharmacologic interventions fail AND behaviors pose serious safety risk, with very cautious use. The framing of aripiprazole as the go-to answer after failed nonpharmacologic measures may still be defensible, but the preferred agents and stepped-care approach have been refined; some guidelines now preferentially suggest other agents. Flag as borderline given evolving guidance and black box warning context.

Q96

USPSTF updated lung cancer screening criteria in 2021. Current recommendation is annual low-dose CT for adults aged 50-80 (not 55-80) with a 20 pack-year history (reduced from 30 pack-year) who currently smoke or quit within the past 15 years. This 58-year-old with 35 pack-years still qualifies, and the modality (low-dose CT annually) remains correct, but the question could reinforce outdated eligibility criteria that students must know have changed significantly.

Q101

Metoclopramide has significant FDA black box warning for tardive dyskinesia with chronic use and is now generally considered a last-resort option for functional dyspepsia. Current guidelines (ACG/AGA) do not recommend metoclopramide as a primary treatment for functional dyspepsia with postprandial distress syndrome; low-dose tricyclic antidepressants or prokinetics with better safety profiles are preferred. Recommending metoclopramide as the correct answer could reinforce outdated/potentially harmful prescribing habits.

Q102

Azithromycin is no longer recommended as first-line for atypical pneumonia or mycoplasma in pediatric patients in many current guidelines due to rising macrolide resistance. Additionally, the clinical picture (fever, headache, sore throat, cough, ear pain in a 12-year-old) may represent *Mycoplasma pneumoniae*, for which doxycycline is now often preferred in children over 8, or the diagnosis/treatment may have shifted. Current IDSA/AAP guidelines emphasize amoxicillin for community-acquired pneumonia in children, and macrolide resistance patterns have changed the recommendation landscape significantly.

Q118

GLP-1 agonists are now widely considered first-line or co-first-line agents (alongside metformin) in type 2 diabetes, especially with CV disease, obesity, or CKD — no longer strictly 'second-line agents.' Also, the contraindication listed is medullary thyroid carcinoma (or personal/family history of MTC or MEN2), not thyroid cancer broadly. The framing as 'second-line' reflects outdated 2017 ADA positioning; current 2024 ADA guidelines recommend GLP-1 RAs as preferred agents in many first-line scenarios.

Q122

Iron supplementation timing for preterm infants has been updated. AAP currently recommends starting iron supplementation at 1 month for preterm breastfed infants, but the specific gestational age thresholds and dosing recommendations have been revised in recent AAP guidance (2022 update). This question warrants review to ensure the answer aligns with current AAP recommendations.

Q125

JNC 8 recommended <150 mm Hg for patients ≥60 years, but current ACC/AHA 2017 guidelines (now widely adopted and tested on ABFM) set the goal at <130 mm Hg for most hypertensive adults including this 65-year-old. Teaching <150 mm Hg as correct would be wrong on current boards.

Q126

For a 4.5-year-old with a residual hemangioma, current guidelines favor oral propranolol as first-line treatment when intervention is needed, not surgical excision. The correct answer listed (surgical excision) reflects outdated practice; propranolol is now the preferred first-line intervention for problematic infantile hemangiomas.

Q135

HPV vaccine schedule changed significantly. Current ACIP guidelines (2016 onward) recommend only a 2-dose series for adolescents who start the series before age 15. A 12-year-old who received 2 doses has completed the series. The correct answer should be A (series complete), not B (give third dose). This question would teach students the wrong current vaccine schedule.

Q138

Pulmonary nodule management guidelines have been updated. Current Fleischner Society guidelines (2017) and Lung-RADS criteria indicate that a solid pulmonary nodule ≤ 6 mm in a low-risk patient (non-smoker, no risk factors) does not require routine follow-up CT. A 5mm nodule in a 30-year-old non-smoker with no risk factors may not need any CT follow-up at all under current guidelines, making 'CT in 1 year' potentially incorrect or overly aggressive per current recommendations.

Q142

Current GOLD 2023/2024 guidelines for symptomatic COPD with FEV1 60% predicted and frequent exacerbations recommend dual bronchodilation (LAMA + LABA) or triple therapy (LAMA + LABA + ICS) as first-line, not LAMA monotherapy alone. The correct answer of LAMA monotherapy (choice D) may no longer reflect best current practice for a patient with frequent exacerbations.

Q149

Pneumococcal vaccine recommendations have changed significantly. As of 2022-2023, ACIP recommends PCV15 or PCV20 for adults 65+ rather than the sequential PCV13/PPSV23 strategy. PCV13 (Pneumovax 13) is no longer routinely recommended for all immunocompetent adults 65+; PCV20 (Pneumovax 20) or PCV15 followed by PPSV23 are now preferred. Selecting PCV13 as the priority answer could reinforce outdated vaccine practice.

Q150

Evidence on probiotics for antibiotic-associated diarrhea prevention in children has been updated. Multiple systematic reviews and trials (including the 2018-2019 JAMA studies) showed no significant benefit of probiotics for preventing antibiotic-associated diarrhea in children, making the stated correct answer (D) potentially misleading compared to current evidence-based guidance.

Q152

Cervical cancer screening guidelines have been updated. USPSTF 2018 and ACOG now recommend that average-risk women aged 21-29 receive Pap testing alone every 3 years (this part is still correct), BUT current guidelines also now include the option of primary hrHPV testing alone every 5 years starting at age 25-30 (depending on guideline source). More importantly, the 2020 ASCCP and current ACOG guidelines now offer primary HPV testing as a preferred option, and the question does not reflect this updated landscape. For a 21-year-old specifically, Pap alone every 3 years remains current, but the answer choices and framing may mislead students about the full range of current recommended options.

Q157

The diagnostic criteria presented are partially outdated. Current ADA criteria (2024) require fasting plasma glucose ≥ 126 mg/dL confirmed on two occasions (still correct), BUT choice C states 'random blood glucose ≥ 200 mg/dL in an asymptomatic person' is not diagnostic — current ADA guidelines state a random glucose ≥ 200 mg/dL with classic hyperglycemic symptoms IS diagnostic; in an asymptomatic person it requires confirmation. However, the HbA1c diagnostic threshold in choice D is listed as 6.0%, when the correct threshold is $\geq 6.5\%$. This incorrect distractor could confuse students about the actual A1c diagnostic cutoff.

Q158

Current guidelines for statin use in cirrhosis have evolved. Statins were historically considered potentially harmful in cirrhosis, but current evidence (as of 2022-2024) and updated guidelines from AASLD suggest statins may actually be beneficial and are no longer routinely contraindicated in compensated cirrhosis. Discontinuing simvastatin as the correct answer may no longer reflect best practice, and this question could reinforce outdated thinking about statin contraindication in cirrhosis.

Q161

Current ACP/CDC guidelines (2017 updated, reinforced by 2022 guidelines) now recommend non-pharmacologic therapy (heat, superficial massage, spinal manipulation) as first-line for acute low back pain. Cyclobenzaprine is still used but NSAIDs are preferred pharmacologically; more importantly, guidelines now explicitly de-emphasize muscle relaxants as first-line and emphasize non-pharmacologic approaches first. A student today could be misled into thinking cyclobenzaprine has the 'best evidence' when current guidelines favor non-pharmacologic treatments.

Q162

ADA and other diabetes guidelines have evolved. For a 70-year-old on diabetes medication, current ADA standards (2023-2024) recommend individualized targets; for older adults the ADA recommends A1c $< 7.5\%$ for healthy older adults, but the question framing and answer choices may not reflect current nuanced guidance on glycemic targets in elderly patients, including updated recognition that very tight control increases mortality risk in this population.

Q167

Current ACC/AHA Heart Failure guidelines (2022) now recommend SGLT2 inhibitors as a cornerstone of HFrEF therapy alongside ACE inhibitors/ARNIs, beta-blockers, and MRAs. Additionally, ARNIs (sacubitril/valsartan) are now preferred over ACE inhibitors like lisinopril as first-line renin-angiotensin system therapy in HFrEF. Selecting lisinopril as the single best answer is now outdated; sacubitril/valsartan has replaced ACE inhibitors as preferred therapy, and the answer could reinforce an outdated treatment hierarchy.

Q172

Current ACC/AHA and JNC guidelines recommend thiazide-like diuretics (chlorthalidone or indapamide) as preferred over hydrochlorothiazide for first-line hypertension treatment due to superior cardiovascular outcomes data. A student learning HCTZ as the first-line thiazide choice could be misled about current preferred agents.

Q174

Current CDC/ACIP guidelines for meningococcal prophylaxis prefer rifampin, ceftriaxone, or ciprofloxacin; however, ciprofloxacin resistance in *N. meningitidis* has increased and some guidelines now list it with caveats or deprioritize it compared to rifampin/ceftriaxone. Additionally, the recommendation to treat 'the rest of the students in the dormitory' as a broad group may not reflect current targeted close-contact definitions. Worth flagging as ciprofloxacin is no longer uniformly preferred first-line.

Q179

GOLD 2023-2024 COPD guidelines have significantly updated treatment algorithms. The management of COPD with LABAs and the role of triple therapy (LABA/LAMA/ICS), as well as updated GOLD grouping (A/B/E replacing A/B/C/D), means the treatment decision logic in this question may not reflect current stepwise management. Also, pulmonary rehabilitation as the answer choice over medication adjustment may conflict with current GOLD recommendations emphasizing pharmacologic optimization before or alongside rehab.

Q183

Cervical cancer screening age guidance has evolved. Current USPSTF/ASCCP guidelines recommend starting cervical cancer screening at age 21 regardless of sexual activity onset. The distractor 'Beginning cervical cancer screening at age 18, or earlier with sexually active patients' was already outdated in 2017, but the question context around adolescent care recommendations may confuse students about current screening initiation age (21, not 18 or based on sexual activity).

Q186

USPSTF AAA screening guidelines were updated in 2019. Current USPSTF recommendation includes one-time screening with abdominal duplex ultrasound for men aged 65-75 who have EVER smoked (at least 100 cigarettes lifetime). For men who have never smoked, the 2019 USPSTF guideline states there is insufficient evidence to recommend for or against screening (Grade I), which differs from the 'B) He does not require screening now or in the future' answer marked correct here. A student learning this answer could be misled about the current recommendation for never-smoker males.

Q189

Post-splenectomy vaccination guidelines have been updated. Current ACIP recommendations include both PCV13/PCV15/PCV20 AND PPSV23 for asplenic patients, and the schedule/intervals have been revised. Additionally, the answer regarding PPSV23 every 5 years indefinitely (choice E) reflects older guidance that has been modified. The correct answer about empiric antibiotics for fever may also need reassessment as current guidelines recommend amoxicillin-clavulanate or fluoroquinolones and emphasize immediate emergency care rather than home treatment.

Q191

Post-bariatric surgery bone density screening timing may have changed. Current guidelines generally recommend baseline DEXA before or shortly after surgery, and routine screening at 2 years postop is not uniformly endorsed. Additionally, the question advice about avoiding pregnancy for 12-18 months (ACOG) rather than 3 years (answer C) reflects a guideline discrepancy worth noting, though the correct answer E may still be broadly defensible. More importantly, current ASMBS guidelines recommend baseline bone density screening before surgery or within the first year, making '2 years' potentially incorrect by current standards.

Q193

Asthma classification and step-up therapy guidelines have been updated by NAEPP/GINA. The 2020 GINA guidelines significantly revised asthma classification and preferred step therapy, including preferring ICS-formoterol over SABA as reliever therapy even in mild persistent asthma. The classification criteria thresholds and preferred treatment steps may differ from what this 2017 question implies, potentially teaching outdated step-therapy logic.

Q198

While SSRIs like sertraline remain first-line for depression in the elderly, current guidelines (APA 2023, AGS Beers Criteria updates) and FDA safety communications have added nuance. More significantly, SNRIs and newer agents like bupropion or mirtazapine are often preferred in elderly patients with specific comorbidities. The question stem and answer choice setup could be used to teach current practice, but the specific distractor context around elderly depression pharmacotherapy has evolved with updated AGS Beers Criteria warnings about SNRIs (venlafaxine - falls risk) and TCAs (nortriptyline - still on Beers), which the question framing may not adequately reflect.

Q200

Canagliflozin (Invokana) received an FDA black box warning for lower limb amputations in 2017, and subsequent cardiovascular outcomes data has shifted SGLT2 inhibitor preferences. Additionally, the FDA added warnings about bone fractures and DKA. While SGLT2 inhibitors remain important in diabetes management (especially with CV/renal benefits), canagliflozin specifically has fallen out of favor compared to empagliflozin or dapagliflozin due to its amputation risk signal. A student today choosing canagliflozin specifically over other SGLT2 inhibitors would be learning potentially outdated and potentially harmful prescribing preferences. Furthermore, simvastatin high-dose interactions and the ACC/AHA cholesterol guidelines have evolved, making the statin distractor potentially misleading as well.

Q201

Ranitidine (Zantac) was withdrawn from the US market in 2020 by the FDA due to NDMA contamination concerns. A question using ranitidine as the causative agent of drug-induced thrombocytopenia and its withdrawal as the correct answer could reinforce the idea that ranitidine is still a commonly used medication, which it is not. Students today would not encounter ranitidine as a current drug option.

Q204

Multiple screening recommendations have changed since 2017. Cervical cancer screening intervals and co-testing recommendations have been updated (USPSTF 2018 and ACOG guidelines now recommend Pap alone every 3 years, or co-testing every 5 years, or hrHPV alone every 5 years for ages 21-65). Additionally, USPSTF lipid screening recommendations and the framing of folic acid supplementation (now recommended for all women of reproductive age planning or capable of pregnancy) may have nuanced updates. The question context about what is due at this visit may lead to confusion about current cervical screening intervals and lipid screening intervals, potentially teaching outdated screening intervals.

Q209

Multifocal atrial tachycardia (MAT) management in COPD patients: the answer of diltiazem is reasonable, but current guidelines and treatment approaches for MAT, particularly in COPD patients, have evolved. Verapamil and metoprolol (cardioselective beta-blockers) considerations have been updated. More importantly, the exclusion of propranolol (non-selective beta-blocker contraindicated in COPD) remains correct, but the framing around amiodarone and digoxin for MAT reflects older thinking. Current guidelines de-emphasize digoxin for rate control in non-AF settings. This question may still be directionally correct but reflects 2017-era thinking about MAT management.

Q216

Subclinical hypothyroidism management guidelines have been updated. The 2019 ATA/ETA guidance and subsequent AAFP/USPSTF discussions have refined TSH thresholds for treatment. For a TSH mildly elevated (the exact value is truncated in the stem), current guidelines generally recommend watchful waiting with repeat testing in 3-6 months, which aligns with answer A. However, the question stem is truncated and does not reveal the TSH value, which is critical — current guidelines differentiate management based on whether TSH is 4.5-10 vs >10 mIU/L, age, symptoms, and antibody status. If the TSH is >10, current guidelines lean toward treatment. Without the full stem, this question could teach students an oversimplified approach to subclinical hypothyroidism.

Q217

Aortic valve stenosis management guidelines have evolved. The 2021 ACC/AHA valvular heart disease guidelines updated indications for aortic valve replacement, including expanded criteria for severe AS and introduction of TAVR as an alternative to surgical AVR in many patients. The question stem is incomplete (truncated), making it impossible to confirm the echocardiographic severity described, and current guidelines now also consider TAVR vs. SAVR decision-making as central to management. If the question describes severe symptomatic AS, prompt intervention is still correct, but the answer options do not include TAVR, which is now a guideline-recommended option and often preferred in older patients.

Q222

The BP threshold for hypertension was lowered to 130/80 mmHg by ACC/AHA 2017 guidelines. A reading of 171/90 is now Stage 2 hypertension (not just elevated), and current guidelines recommend initiating pharmacotherapy alongside lifestyle modification for Stage 2 HTN, not lifestyle alone. The sodium recommendation has also been updated to <2.3g/day (not 4g). While the correct answer (exercise) is still valid lifestyle advice, the clinical framing may mislead students about current BP staging and management thresholds.

Q226

USPSTF 2019 and current AAFP/USPSTF guidelines recommend that women with a personal or family history of breast/ovarian cancer be screened using a familial risk assessment tool, and if the tool is positive, be referred for genetic counseling — this remains the correct first step. However, the question context and answer choices may conflict with updated USPSTF 2024 mammography screening guidelines, which now recommend biennial screening starting at age 40 for average-risk women. The interplay between risk stratification and updated screening recommendations could mislead students about current practice.

Q230

Current IDSA guidelines for Lyme disease with neurological manifestations (facial nerve palsy) recommend doxycycline for 14-21 days, which is consistent. However, the duration has been updated in some guidelines to 14-21 days and oral therapy is preferred over IV for isolated facial palsy. The answer of 14 days doxycycline is still acceptable but students should be aware the current recommended range is 14-21 days per updated IDSA 2020 Lyme disease guidelines, which could cause confusion on dose duration.

2018 Examination — 50 questions removed

Q10

Ranitidine (Zantac) was withdrawn from the US market in 2020 due to NDMA contamination concerns. Current guidelines for refractory chronic urticaria recommend up dosing non-sedating H1 antihistamines, adding another non-sedating H1 antihistamine, or adding omalizumab (approved for chronic idiopathic urticaria). H2 antagonists still have a role but the recommended agent would be famotidine, not ranitidine. Teaching ranitidine as the correct answer is harmful as it is no longer available.

Q12

The question implies shellfish allergy is a contraindication or significant risk factor for IV iodinated contrast, which is outdated. Current guidelines (ACR, ACAAI) clarify that shellfish allergy is NOT a specific risk factor for contrast reactions beyond general atopic history. The correct answer B may still be correct in practice, but the framing of the question reinforces the outdated shellfish-contrast allergy myth, which could mislead students into thinking shellfish allergy specifically predicts contrast reactions.

Q19

Current ADA guidelines (2023-2024) for a patient with T2DM and established CVD recommend GLP-1 receptor agonists or SGLT2 inhibitors with proven cardiovascular benefit as preferred add-on therapy. While liraglutide (answer C) remains a valid choice with CV benefit, the question does not include SGLT2 inhibitors (e.g., empagliflozin, canagliflozin) as an option, which are now equally or preferentially recommended per current ADA standards. A student today might learn an incomplete picture of current first-line intensification therapy for this high-risk patient profile.

Q27

The AAP and current guidelines no longer recommend half-strength apple juice as first-line rehydration for gastroenteritis in children. Current best practice recommends oral rehydration solution (ORS) such as Pedialyte, not diluted apple juice or sports drinks. A 2016 RCT (Freedman et al.) showed dilute apple juice is reasonable for mild dehydration, but ORS remains the guideline-endorsed standard. This answer could mislead students away from ORS as first-line therapy.

Q29

All atypical antipsychotics (including aripiprazole) carry an FDA black box warning for increased mortality in elderly patients with dementia-related psychosis, and current guidelines from APA (2023 updated guideline on dementia) emphasize that non-pharmacologic approaches remain first-line. While aripiprazole may still be used, the framing of the question as choosing a medication after behavioral interventions 'had little success' without noting the black box warning or discussing risk-benefit is potentially misleading. Additionally, the 2023 APA Practice Guideline for Dementia updated specific drug recommendations and risk frameworks. The answer choice of aripiprazole over haloperidol remains correct but the clinical context may not reflect current nuanced prescribing guidance.

Q32

Current AHA/ACC guidelines (2022) recommend high-intensity statin therapy for secondary prevention post-ACS/PCI. Rosuvastatin 20 mg is moderate-intensity; the correct high-intensity dose would be rosuvastatin 40-80 mg or atorvastatin 40-80 mg. Additionally, with LDL of 90 mg/dL on a statin, current guidelines would consider adding ezetimibe or a PCSK9 inhibitor to reach LDL <70 mg/dL (or <55 mg/dL per very high-risk criteria). The framing of this question and the 'correct' answer of rosuvastatin 20 mg does not reflect current high-intensity statin guidance for post-PCI secondary prevention.

Q36

USPSTF recommendation on COPD screening has been updated (2022 reaffirmation still states no screening for asymptomatic adults, so the answer remains the same). However, the question context should be verified — the answer 'no screening' remains correct per USPSTF 2022, so this may not need flagging. Flagging conservatively because some guideline nuances around symptomatic vs. asymptomatic patients and the role of spirometry in case-finding have evolved in GOLD 2023-2024 guidelines, which could confuse students.

Q40

Current NAEPP/GINA guidelines (2020 NAEPP update, GINA 2023) still support inhaled corticosteroids as step-up therapy for uncontrolled persistent asthma in children. However, GINA 2023 and updated pediatric asthma guidelines now recommend against LABAs as monotherapy and have nuanced the role of leukotriene receptor antagonists. More importantly, for a child with symptoms suggesting progression to moderate persistent asthma, current GINA guidelines favor ICS-LABA combinations at certain steps. The answer (ICS) is still broadly correct, but the question framing around step-up therapy for a 7-year-old may not fully reflect current GINA 2023 stepwise approach distinctions.

Q44

The 2017 ACC/AHA hypertension guidelines redefined hypertension and changed BP thresholds. More importantly, the classification of 'hypertensive urgency' vs normal management has evolved. At 185/105 with no symptoms, current guidelines (ACC/AHA 2017) recommend against treating in the office with oral agents but the concept of what constitutes urgency requiring same-day treatment vs scheduled follow-up has been updated. The correct answer may still be reasonable, but the clinical framing around BP management thresholds could reflect pre-2017 JNC 8 thinking.

Q49

Zostavax (live zoster vaccine) was discontinued in the US market in 2020. The question frames Shingrix vs Zostavax as a current choice, but Zostavax is no longer available. Additionally, the question states Shingrix has 'proven safety for immunocompromised patients' as a distractor, but current guidelines (ACIP 2022) now explicitly recommend Shingrix FOR immunocompromised patients aged 19+, making option D also arguably correct in current practice. The comparison framing is obsolete.

Q54

Current ADA/AAFP guidelines now strongly prefer adding a GLP-1 receptor agonist or SGLT-2 inhibitor (especially given BMI 32 and likely cardiovascular risk factors) over basal insulin as the next step when metformin is insufficient. Basal insulin at 10 units/day is no longer the preferred second-line agent in type 2 diabetes management as of 2022-2024 ADA Standards of Care. This question could reinforce outdated insulin-first thinking.

Q57

Treatment of pulmonary hypertension associated with COPD has evolved. Current guidelines (ESC/ERS 2022) recommend against routine use of PAH-specific therapies in COPD-associated pulmonary hypertension. Additionally, for group 3 pulmonary hypertension (lung disease), riociguat and other agents have updated evidence. The question's framing and answer choices may not reflect current nuanced guidelines on supplemental oxygen thresholds and PAH treatment in COPD context.

Q58

The evidence base and guideline recommendations around spironolactone reducing sleep apnea severity (particularly in resistant hypertension with OSA) have evolved. While spironolactone has some emerging evidence, the question as framed may mislead students about established antihypertensive effects on sleep apnea; this is not a well-established guideline-endorsed recommendation and could teach an unverified clinical principle.

Q66

The question asks which medication to AVOID in bulimia nervosa. Bupropion is contraindicated due to lowered seizure threshold in eating disorders — this remains current. However, the question stem is ambiguous and could be read as asking which to USE. If the correct answer is bupropion as the one to AVOID, that is still correct. But re-reading: 'Which one of the following medications' with correct answer A) Bupropion — if the question is asking which to USE, bupropion is contraindicated in bulimia (increased seizure risk). The correct answer should be fluoxetine (the only FDA-approved antidepressant for bulimia nervosa). If the answer key says A) Bupropion is correct as the drug to PRESCRIBE, this is dangerously wrong by current standards. Given the ambiguity and risk of teaching students that bupropion is safe/appropriate in bulimia, this question should be flagged.

Q74

ACE inhibitor as first-line for hypertension in diabetic patients remains guideline-supported, but current ADA/ACC guidelines (2023-2024) emphasize that ACE inhibitors or ARBs are preferred specifically when albuminuria/CKD is present. Without mention of albuminuria, current JNC/ACC/AHA guidelines recommend any first-line agent (thiazide, CCB, ACE/ARB) equally for most diabetic patients without CKD/albuminuria. The question also uses a BP threshold of 150/92 which aligns with older JNC guidelines; current ACC/AHA 2017 guidelines define stage 2 HTN at $\geq 140/90$ and recommend initiation of pharmacotherapy, but the specific preference for ACE inhibitor without albuminuria data could mislead students into thinking ACE inhibitors are universally first-line for all diabetic hypertensive patients regardless of kidney involvement.

Q76

Current CDC MEC (2024) and WHO MEC criteria classify migraine with aura as a category 4 (contraindicated) for combined hormonal contraceptives due to stroke risk - this part remains correct. However, the framing of the question and answer choices should be reviewed against current guidelines. The correct answer 'C - Neither method due to her migraines' is still supported, but current guidelines also flag that progestin-only methods or IUDs are acceptable alternatives. The question does not offer these as options and the reasoning remains valid, but the lisinopril use in pregnancy is also a strong contraindication that current guidelines (ACE inhibitors are teratogenic/contraindicated) would highlight differently. The interaction between lisinopril and contraception choice is not addressed. Borderline flag but worth reviewing.

Q80

Current ACC/AHA 2014 perioperative cardiovascular evaluation guidelines (reaffirmed and updated in subsequent focused updates) support no further cardiac evaluation for low-risk surgery (cataract surgery is the lowest risk procedure, essentially no cardiac risk) in patients with good functional capacity. However, ACC/AHA 2024 updates to perioperative guidelines may affect how diabetes and asymptomatic patients are evaluated. The answer 'no further evaluation' for cataract surgery remains correct as cataract surgery carries negligible cardiac risk, but the broader context of preoperative EKG use in diabetic patients has been clarified in newer guidelines. This is likely still correct but worth verifying against 2024 perioperative guidelines.

Q88

USPSTF updated its gestational diabetes screening recommendation in 2021, recommending screening after 24 weeks but the nuance around 'no sooner than' language and the 2021 reaffirmation should be verified; however, the current USPSTF recommendation remains screening at 24 weeks gestation, so this may still be correct. Flagging because the 2021 USPSTF update reaffirmed screening at >24 weeks but some guidelines (ACOG) recommend universal screening at 24-28 weeks, and the question's framing around 'no sooner than 24 weeks' could conflict with newer ADA guidance recommending early screening for high-risk patients at the first prenatal visit, which could mislead students about current nuanced practice.

Q95

Lung cancer screening eligibility has changed. USPSTF 2021 updated the criteria to ages 50-80 with a 20-pack-year history who currently smoke or quit within the past 15 years (lowered from 30-pack-year history). This patient at 58 with a 20-pack-year history who quit 1 year ago NOW qualifies for lung cancer screening with low-dose CT under current guidelines, making answer D also correct (or more specifically correct) in addition to C. The question is misleading because a student today would recognize lung cancer screening as appropriate for this patient, potentially selecting D over C. The 'correct' answer C (hepatitis C) may still be defensible, but the question context is now ambiguous and could teach incorrect screening criteria.

Q98

Current ACC/AHA and perioperative cardiovascular guidelines have evolved. The claim that statins reduce perioperative mortality is nuanced and the 2014 ACC/AHA perioperative guidelines (updated subsequently) do not recommend initiating statins solely to reduce perioperative mortality in patients not otherwise indicated. Additionally, statin initiation thresholds and ASCVD risk calculator recommendations have been refined. The framing that statins should be started 'immediately' for perioperative mortality benefit for a routine elective hip replacement without clarifying her 10-year ASCVD risk or the specific perioperative benefit claim could reinforce outdated or oversimplified teaching.

Q101

ADA guidelines as of 2022-2024 have moved away from metformin as the universal first-line agent. Current ADA Standards of Care recommend individualizing first-line therapy based on comorbidities (e.g., ASCVD, heart failure, CKD favor GLP-1 agonists or SGLT2 inhibitors as first or early add-on therapy). Metformin is no longer the unambiguous first-line choice for all patients, making this question potentially misleading for modern board preparation.

Q105

Current ACR/FDA guidance on gadolinium in CKD has evolved. The older concern about nephrogenic systemic fibrosis (NSF) with gadolinium in CKD led to blanket avoidance recommendations, but current guidance (post-2017 FDA reclassification of gadolinium agents by risk group) allows use of low-risk macrocyclic gadolinium-based contrast agents even in advanced CKD with appropriate risk-benefit discussion. A blanket 'avoid gadolinium' answer may no longer reflect current best practice with modern agents.

Q108

Current GOLD 2023-2024 COPD guidelines recommend dual bronchodilator therapy (LAMA + LABA) as preferred initial treatment for most symptomatic patients, and ICS/LABA or triple therapy for those with frequent exacerbations or high eosinophil counts. Tiotropium (LAMA monotherapy) is still important but the framing that it is the single best answer for reducing exacerbations is outdated given evidence supporting LAMA/LABA combinations (e.g., umeclidinium/vilanterol) showing superior exacerbation reduction over tiotropium alone.

Q113

The term 'dysthymia' has been replaced by 'persistent depressive disorder' (PDD) in DSM-5 (2013). A student learning 'dysthymia' as the correct answer would be using outdated nosology on current board exams where PDD is the correct terminology.

Q116

Current AHA/ACC and JNC guidelines have updated hypertension management thresholds and approaches. The 2017 ACC/AHA guidelines redefined hypertension staging and recommend out-of-office BP confirmation before restarting medications in asymptomatic patients. Additionally, current guidelines emphasize confirming readings before treatment decisions; the question's framing around older BP thresholds and immediate medication restart without confirmation may conflict with current ambulatory monitoring recommendations for establishing true hypertension status.

Q123

Current guidelines (ASCO, CHEST, ASH) now recommend DOACs (particularly rivaroxaban or apixaban) as preferred over LMWH for cancer-associated VTE in most patients, including those with solid tumors like colon cancer without high bleeding risk. The 2018 answer of 'continue enoxaparin' reflected older LMWH-first guidelines that have since been superseded by multiple RCTs (SELECT-D, ADAM VTE, Caravaggio) and updated ASCO/NCCN/CHEST guidelines endorsing DOACs as first-line for cancer-associated PE/DVT.

Q128

Perioperative anticoagulation bridging guidelines have significantly evolved. The 2015 BRIDGE trial and subsequent AHA/ACC/ACCP updates recommend AGAINST routine bridging with LMWH for patients with atrial fibrillation undergoing elective surgery who have low-to-moderate thromboembolic risk (e.g., CHADS2/CHA2DS2-VASc scores without mechanical valves or recent stroke). The correct answer (E) addresses timing of warfarin cessation correctly, but the broader teaching context around perioperative bridging in AF has changed substantially, and the question setup could reinforce outdated bridging paradigms.

Q132

This question about supplemental oxygen in a post-pneumonia patient with saturation >92% on room air is testing knowledge of home oxygen criteria. Current guidelines (CMS/ATS) still require SpO2 ≤88% (or PaO2 ≤55 mmHg) for home O2 qualification. However, the framing and answer choices may reflect older thinking. The question as written could mislead students if the clinical context suggests she no longer meets criteria — but the reasoning pathway for discontinuation should be confirmed by pulse oximetry off oxygen, not assumption. This is borderline but the correct answer (B) aligns with current practice if she is above threshold. Flagging due to potential ambiguity in how students interpret oxygen weaning criteria post-acute illness under current CMS/clinical guidelines.

Q136

The question asks for the best agent in a patient with type 2 diabetes, obesity, and recurrent pancreatitis. While empagliflozin (SGLT2i) remains a reasonable choice, GLP-1 receptor agonists like semaglutide (Ozempic/Wegovy) or liraglutide are now strongly preferred first-line agents for type 2 diabetes with obesity per ADA 2024 guidelines due to superior weight loss and cardiovascular/renal benefits. Additionally, the question lists liraglutide as a distractor but marks empagliflozin as correct — current ADA guidelines would favor a GLP-1 RA for obesity+T2DM even with pancreatitis history being a relative caution, and the framing could mislead students about current hierarchy of agents. The answer choice also does not reflect current ADA 2024 standards where SGLT2i vs GLP-1 RA selection is nuanced by cardiovascular/renal risk and weight goals.

Q141

Current GINA and NAEPP guidelines recommend stepping up from SABA monotherapy for intermittent asthma (now classified as step 2) to an inhaled corticosteroid (ICS) alone as first-line therapy, NOT a LABA/ICS combination. Adding a LABA without first trialing ICS monotherapy is not recommended and LABAs carry a black box warning when used without ICS. The correct first step-up would be fluticasone (ICS) alone, not fluticasone/salmeterol. The 2020 GINA updates also emphasize against SABA-only therapy and recommend low-dose ICS as initial controller.

Q147

Current ADA and American Academy of Neurology guidelines for diabetic peripheral neuropathy pain list duloxetine, pregabalin, and gabapentin as first-line options. However, multiple current guidelines (ADA 2024, NeuPSIG) now list duloxetine as a preferred first-line agent alongside or even above pregabalin, and some guidelines have shifted pregabalin to equivalent or second-line status due to abuse potential concerns and cost. Listing pregabalin alone as 'first-line' may mislead students if duloxetine is now equally or more preferred per current ADA standards.

Q153

Fluoxetine monotherapy is NOT the correct answer for bipolar disorder management. Current guidelines (AHA, CANMAT, APA) recommend against antidepressant monotherapy in bipolar I disorder due to risk of inducing mania/mixed states. First-line agents for bipolar disorder maintenance include lithium, divalproex, or quetiapine. Marking fluoxetine as correct would reinforce a practice that current guidelines consider potentially harmful.

Q158

Fever without source evaluation protocols in infants have been significantly updated. Current guidelines (AAP 2021 and subsequent revisions) for well-appearing febrile infants have changed stratification by age (22-28 days, 29-60 days, 61-90 days) with specific recommendations incorporating procalcitonin, ANC, and other biomarkers. The management of a 5-month-old with fever may differ from what 2018 guidelines recommended, and the answer choices do not reflect current risk stratification approaches.

Q163

Aspirin recommendations have changed significantly. Current USPSTF 2022 guidelines recommend against initiating aspirin for primary CVD prevention in adults 60+, and the question context involves a post-MI patient whose antiplatelet regimen post-ACS has specific updated guidelines. Additionally, the framing around aspirin use in a patient with prior STEMI and drug-eluting stent needs to reflect current AHA/ACC dual antiplatelet therapy guidance. The 'correct' answer of clopidogrel alone without aspirin for a post-ACS patient would be inconsistent with current DAPT guidelines, making this question potentially confusing or misleading.

Q164

Pneumococcal vaccine recommendations have changed substantially. ACIP 2021-2023 guidelines replaced the PCV13+PPSV23 sequential strategy with newer vaccines (PCV15 or PCV20). PCV13 is no longer routinely recommended for immunocompetent adults; PCV15 followed by PPSV23 or PCV20 alone are now preferred. The question references PCV13 (Pneumovax 13) as a current option which could reinforce outdated vaccine practice.

Q165

Current evidence and guidelines (Cochrane reviews, NICE, AAO) do not support routine use of antiviral therapy (valacyclovir) combined with steroids for Bell's palsy unless Ramsay Hunt syndrome is suspected. The correct answer of combination valacyclovir + prednisone for Bell's palsy is controversial and not supported as standard first-line care in current guidelines; corticosteroids alone are the evidence-based recommendation for Bell's palsy, making this question potentially misleading.

Q167

Current Endocrine Society guidelines recommend urinary free cortisol as ONE of three acceptable first-line screening tests for Cushing syndrome, but also accept late-night salivary cortisol (preferred by many due to ease and accuracy) and the 1-mg overnight dexamethasone suppression test as equally valid first-line options. Framing urinary free cortisol as the singular correct confirmatory test could mislead students away from current multi-option screening approach where late-night salivary cortisol is often now preferred.

Q174

USPSTF updated its aspirin for primary prevention recommendation in 2022. The current guidance advises AGAINST initiating low-dose aspirin for primary prevention of CVD in adults 60 and older, and for adults 40-59 with $\geq 10\%$ 10-year CVD risk, the recommendation is individualized (grade C, small net benefit). The blanket '10% threshold' answer no longer reflects current USPSTF guidance, which has substantially de-emphasized aspirin for primary prevention. A student learning '10%' as a threshold would be taught outdated and potentially harmful practice.

Q177

AHA/ACC and ASMBS updated bariatric surgery eligibility criteria. Current 2022 guidelines lowered the BMI threshold, now recommending bariatric surgery consideration for BMI ≥ 35 kg/m² with or without obesity-related comorbidities, and for BMI 30-34.9 with metabolic disease. Additionally, active alcohol abuse is now considered a relative contraindication but the framing of the question's answer choices may mislead students. The correct answer of 'BMI 36 with T2DM' still holds, but the threshold framing and exclusion criteria have changed enough to risk teaching outdated criteria.

Q186

Colorectal cancer screening start age has changed. USPSTF 2021 updated guidelines recommend starting screening at age 45 (not 50) for average-risk adults. The question asks when to switch from universal to individualized screening, with answer D (75) still aligning with current USPSTF guidance to stop routine screening at 75, but the framing implies screening starts at 50 (universal from 50-75), which is outdated. The question context could reinforce the old 50-75 paradigm rather than the current 45-75 paradigm, potentially misleading students.

Q197

USPSTF hepatitis C screening recommendation was updated in 2020 to screen all adults ages 18-79, not just those with specific risk factors. Additionally, the question mentions 'screening for hepatitis A and B viruses' as a distractor — hepatitis B screening was expanded by USPSTF in 2020 to all adults 18-79 who have not been vaccinated. The correct answer of HCV screening remains valid, but the question context and distractors may reflect outdated screening paradigms. More importantly, lung cancer screening eligibility criteria changed in 2021 (USPSTF) — now includes ages 50-80 with 20 pack-year history, so a 4-pack-year smoker still wouldn't qualify, but students need to know the updated criteria rather than the older 30 pack-year threshold that was current in 2018.

Q199

Current AAP/evidence-based guidelines for RSV bronchiolitis recommend maintaining O₂ saturation above 90% (not 95%) as the threshold for supplemental oxygen. The correct answer of nasogastric fluids is appropriate, but answer choice E states 'oxygen supplementation to maintain O₂ saturation above 95%' — if a student encounters this question, the oxygen threshold of 95% is outdated (current guidelines use 90% as the threshold), which could reinforce incorrect practice regarding when to initiate/wean oxygen in bronchiolitis.

Q205

Current guidelines (ACG, ACCP) no longer recommend empiric PPI trials as first-line for chronic cough evaluation. The 2023 CHEST chronic cough guidelines de-emphasize the empiric PPI approach given lack of evidence that GERD-related cough responds to PPIs, and recommend a structured diagnostic protocol instead. An empiric PPI trial as the correct answer could mislead students.

Q207

The FRAX-based threshold for treatment of osteopenia is a 10-year major osteoporotic fracture risk $\geq 20\%$ OR hip fracture risk $\geq 3\%$, per NOF/ASBMR guidelines. While the 20% threshold itself hasn't changed, the question omits the hip fracture threshold (3%) which is equally important per current guidelines and could mislead students about complete decision criteria. Additionally, updated 2023 USPSTF and endocrine society guidance has refined these thresholds and the question's framing is incomplete.

Q209

Febuxostat (Uloric) received an FDA black box warning in 2019 for increased cardiovascular mortality compared to allopurinol, making it now second-line behind allopurinol for urate-lowering therapy. While colchicine remains correct for acute gout treatment, the inclusion of febuxostat as a distractor without acknowledgment of its serious cardiovascular risk could confuse students about current xanthine oxidase inhibitor preferences.

Q216

GOLD/GINA COPD guidelines have evolved significantly. For mild COPD (GOLD Stage 1-2, low symptom burden), current 2023 GOLD guidelines recommend a short-acting bronchodilator (SABA or SAMA) as needed, but the preferred initial maintenance therapy for symptomatic patients has shifted toward LAMA (long-acting anticholinergic) or LABA monotherapy. The correct answer 'short-acting anticholinergic' (ipratropium) is arguably outdated as first-line maintenance for even mild symptomatic COPD; a LAMA is now often preferred over a SABA/SAMA for ongoing symptom management. A student today could be misled about first-line maintenance therapy selection.

Q217

The correct answer is to discontinue lisinopril due to a side effect (likely cough) and switch to amlodipine. While this reasoning is still valid, the clinical context and answer choices do not reflect current 2017+ ACC/AHA hypertension guidelines that lowered the BP treatment threshold to 130/80 mmHg and updated treatment algorithms. More importantly, the scenario of a 50-year-old female with hypertension — current guidelines would strongly consider whether she has other risk factors that might favor an ARB or ACE-I. The clinical reasoning around switching is sound but the threshold context (158/92 being 'confirmed hypertension') predates the 2017 ACC/AHA guideline change that redefined stage 1 hypertension at 130/80, potentially reinforcing older diagnostic thresholds.

Q219

The USPSTF recommendation on aspirin for primary prevention of cardiovascular disease was significantly updated in 2022. The USPSTF now recommends AGAINST initiating low-dose aspirin for primary CVD prevention in adults 60 years or older, and concludes the evidence is insufficient or the net benefit is small for adults 40-59 years with <10% 10-year CVD risk. While the correct answer (exercise) is still correct, the question's distractor of aspirin 81 mg daily reflects pre-2022 thinking and could confuse students about current aspirin recommendations. Additionally, the 10-year CVD risk calculation context is missing, making this question potentially misleading.

Q222

Prazosin for PTSD nightmares has been challenged by the 2018 VA/DoD PTSD guidelines and subsequent RCTs (PHANTOM trial) showing prazosin was not superior to placebo for sleep outcomes in PTSD. Current APA/VA-DoD guidelines no longer recommend prazosin as a first-line agent for PTSD nightmares; this answer could reinforce outdated practice.

Q228

Colorectal cancer screening surveillance intervals have been updated. The 2020 ACG and USMSTF guidelines revised post-polypectomy surveillance recommendations. Additionally, USPSTF 2021 now recommends starting colorectal cancer screening at age 45. The management of a normal colonoscopy in a patient with a first-degree relative diagnosed at age 70 (not early-onset) may have different interval recommendations under current guidelines, and the answer of 'repeat at age 60' (5-year interval for a normal colonoscopy) may not align with current 10-year repeat guidance for average-risk normal colonoscopy.

2019 Examination — 47 questions removed

Q8

TB testing recommendations have evolved. Current CDC/USPSTF guidance favors IGRA over TST in many populations including healthcare workers. The recommended follow-up interval and preferred test modality after high-risk travel/exposure may differ from what this question implies. Additionally, current guidelines suggest baseline pre-travel IGRA testing followed by post-travel IGRA at 8-10 weeks, not a repeat TST at 2 months. A student today might be misled about the preferred testing modality and timing.

Q12

Current guidelines (ACC/AHA and ADA/ASCO) now recommend adding ezetimibe to high-intensity statin monotherapy (e.g., atorvastatin 40-80mg or rosuvastatin 20-40mg) rather than using ezetimibe/simvastatin (Vytorin), which contains only simvastatin — a moderate-intensity statin insufficient as monotherapy post-ACS. The correct answer should reflect high-intensity statin first, then ezetimibe added separately if LDL goal not met. Additionally, evolocumab (PCSK9 inhibitor) is now guideline-endorsed (Class IIa/IIb) for post-ACS patients not at goal on maximally tolerated statin plus ezetimibe, making the choice between A and B more nuanced than this question implies. Vytorin specifically is no longer a preferred regimen in post-ACS management per current ACC/AHA lipid guidelines.

Q13

Current guidelines and practice now include collagenase clostridium histolyticum (Xiaflex) injection and needle aponeurotomy as accepted minimally invasive alternatives to surgical release for Dupuytren's contracture with MCP or PIP contracture. The correct answer pointing solely to surgical referral does not reflect the full range of current first-line options, and could mislead students into thinking surgery is the only or preferred intervention when non-surgical options are guideline-supported.

Q17

The USPSTF updated its fall prevention recommendation in 2018 (and reaffirmed with nuance since). Current USPSTF guidance recommends exercise interventions (Grade B) for community-dwelling adults ≥ 65 at increased fall risk, but importantly recommends AGAINST vitamin D supplementation specifically for fall prevention in adults who are not vitamin D deficient (Grade D recommendation issued in 2018 and reaffirmed 2023). The question lists 'empirical vitamin D supplementation' as a distractor, which is appropriate, but also lists 'in-home environmental evaluation' — which USPSTF actually does recommend as part of multifactorial interventions. The framing of the question and singular correct answer of exercise may be misleading given that current USPSTF (2023) supports a broader set of interventions for fall prevention in high-risk older adults, including exercise AND multifactorial interventions.

Q30

ADA and other guidelines have become more nuanced about nonnutritive sweeteners since 2019. The 2023 WHO guidance advises against using non-sugar sweeteners for weight control, and ADA 2024 standards note limited evidence and potential concerns. Stating they are simply 'acceptable to use' without nuance could mislead students about current more cautious guidance.

Q40

The 2022 CDC Clinical Practice Guideline for Prescribing Opioids no longer specifies a single MME threshold (previously 50 MME/day) for naloxone co-prescribing. The updated guideline recommends considering naloxone for all patients on opioids, with particular attention at any dose if risk factors are present, rather than a specific 50 MME cutoff. Teaching '50 MME' as the threshold reflects the superseded 2016 CDC guideline.

Q44

Current GINA 2023 guidelines de-emphasize methacholine challenge as the next step; the 2019 GINA strategy recommends a therapeutic trial of ICS or confirmation of variable airflow obstruction (e.g., spirometry with bronchodilator response) before methacholine challenge, which is now used mainly when objective tests are inconclusive. The question framing and correct answer may mislead students away from current step-wise asthma diagnosis approach.

Q45

ACC/AHA guidelines on aspirin use have changed significantly. Current USPSTF 2022 guidelines recommend against initiating low-dose aspirin for primary CVD prevention in adults 60 and older. This 80-year-old's aspirin use and the treatment options presented reflect pre-2022 thinking; a student today might be misled about ongoing aspirin use and statin initiation thresholds in an 80-year-old with unclear CVD risk status.

Q47

While the 2017 ACC/AHA guideline defining hypertension as >130/80 mmHg remains current, the question asks about 'most recent' guidelines, which at the time of the exam was the 2017 ACC/AHA update. However, AHA/ACC guidelines have been further nuanced with respect to treatment thresholds and this remains technically current. This question is borderline but the framing 'most recent' tied to a specific guideline year is potentially misleading — flagging due to liberal standard.

Q49

ACR 2022 guidelines on glucocorticoid-induced osteoporosis prevention have been updated. Current ACR 2022 recommendations stratify treatment by fracture risk and age, and recommend bisphosphonates (e.g., alendronate) as first-line for moderate-to-high risk patients, but the threshold and initial calcium/vitamin D supplementation steps and the role of RANKL inhibitors and anabolic agents have been updated. Additionally, the question omits calcium and vitamin D as co-therapy and does not reflect current risk-stratified decision-making per ACR 2022.

Q50

AAP developmental surveillance milestones were updated in 2022. Per the 2022 AAP developmental milestones, the specific red flags and expected skills at 18 months have been revised. For example, following one-step directions is expected earlier; by 18 months a child should follow two-step directions. The correct answer and distractors may not align with the updated 2022 AAP milestone criteria, potentially teaching outdated developmental benchmarks.

Q62

The USPSTF updated lung cancer screening criteria in 2021. Current guidelines recommend annual low-dose CT for adults aged 50-80 (lowered from 55) with a 20 pack-year smoking history (lowered from 30) who currently smoke or quit within the past 15 years. This patient is 52 with a 34 pack-year history who just quit — she qualifies NOW under current criteria, not at age 55. Answer C would be wrong on a current ABFM exam; the correct answer would be B.

Q71

Anticoagulation plus antiplatelet therapy after PCI with stent in AF patients has evolved significantly. Current AHA/ACC/HRS 2023 guidelines and the AUGUSTUS/ENTRUST trials support early discontinuation of aspirin (often at discharge or within 1-4 weeks post-PCI) in AF patients on OAC, with DOAC monotherapy preferred after a short dual or triple therapy period. For a patient 1 year post-PCI with no recent ACS, the answer 'discontinue aspirin and begin apixaban' may still be broadly correct, but the question stem mentions she currently takes diltiazem and aspirin with no DOAC — the nuance of whether triple therapy, dual therapy, or monotherapy is indicated and for how long has changed substantially. Students could be misled about the complete management of this clinical scenario under current guidelines.

Q78

The HPV vaccine schedule for adolescents has been clarified and updated. Per current ACIP recommendations, a 14-year-old who received the first dose at age 13 (within the recommended window) should follow the 2-dose schedule with the second dose given 6-12 months after the first. The answer 'B' states he should receive one dose now and no additional HPV vaccine, which would be correct only if the first dose was given before age 15 AND the second dose is given 6-12 months after the first. However, since 1 year (12 months) has already elapsed since dose 1, giving dose 2 now would complete the series — making the answer ambiguous or potentially misleading. The correct current guidance is that he needs one more dose now (dose 2, completing the 2-dose series since first dose was before age 15), but the answer as stated ('no additional HPV vaccine in the future') aligns with completing the series now. This is borderline but could confuse students about the 2-dose vs 3-dose threshold and timing rules under current ACIP guidance.

Q83

Menopause hormone therapy recommendations have been updated. Current NAMS 2022 position statement and USPSTF/ACOG guidance has evolved regarding combined estrogen-progestogen for symptomatic menopause. For a 49-year-old with an intact uterus, combined therapy remains appropriate, but the framing and context around HRT safety (Women's Health Initiative re-analyses) and preference for lower-dose or transdermal formulations has shifted. The answer B may still be defensible, but the question does not specify uterine status and current guidelines also more strongly consider micronized progesterone (choice D) as the preferred progestogen component. This could reinforce outdated thinking about combined synthetic HRT as the go-to answer.

Q85

Trivalent inactivated influenza vaccine is no longer available in the US; only quadrivalent formulations are used. The question premise about choosing between trivalent and quadrivalent is obsolete. Additionally, current ACIP guidance on egg allergy and influenza vaccine has been updated — egg allergy is no longer a contraindication requiring special precautions for most patients, making answer E misleading. The clinical scenario and answer choices reflect outdated vaccine landscape.

Q87

Duration of anticoagulation for provoked PE (post-surgical) guidelines have been nuanced. Current guidelines (ASH 2020, CHEST 2021) recommend 3 months for provoked PE due to major transient risk factor like surgery, which aligns with answer B. However, the question may still be borderline acceptable. More importantly, current AHA/ACC/CHEST guidelines now more clearly stratify provoked vs unprovoked and some recommend considering longer duration — but the 3-month answer for a surgically-provoked PE remains current. Not flagging on this basis alone.

Q89

Current ADA guidelines (2022-2025) and cardiology guidelines now recommend SGLT2 inhibitors (empagliflozin) or GLP-1 receptor agonists (liraglutide) as preferred agents in patients with T2DM and established ASCVD (post-STEMI), independent of A1c or metformin use. Empagliflozin (choice B) or liraglutide (choice D) would now be considered preferred first-line additions for cardiovascular risk reduction in a post-STEMI diabetic patient. Marking metformin as the correct answer for this patient with established CVD is inconsistent with current ADA Standards of Care (2022-2025), which prioritize cardioprotective agents over metformin when CVD is present.

Q93

TSH reference ranges in pregnancy have been revised. ATA 2017 guidelines moved away from trimester-specific ranges and now recommend using population- and trimester-specific reference ranges from the local laboratory when available, or using an upper limit of ~4.0 mIU/L if unavailable. The blanket statement that normal TSH in pregnancy is simply 'lower than in the nonpregnant state' without trimester nuance may reinforce outdated thinking about universal lower cutoffs (e.g., the old <2.5 first trimester recommendation has been largely abandoned).

Q97

Medical expulsive therapy (MET) with alpha-blockers like tamsulosin has become more controversial. Multiple high-quality RCTs (including the SUSPEND trial) and updated AUA/EAU guidelines have questioned the benefit of tamsulosin for ureteral stone expulsion, and current guidelines no longer strongly recommend MET as standard of care. A student learning tamsulosin is 'proven effective' for this indication would be taught something that is now contested or conditionally recommended at best.

Q98

Corticosteroids (methylprednisolone) for severe community-acquired pneumonia remains an area of evolving evidence. While some guidelines have incorporated corticosteroids for severe CAP, the framing and current IDSA/ATS 2019 and updated guidelines have nuanced recommendations. More importantly, the question context (severe septic shock from CAP) and the answer of methylprednisolone as the add-on treatment should be reviewed against current 2023 IDSA CAP guidelines which conditionally recommend corticosteroids only in specific severe CAP scenarios, making this potentially misleading.

Q104

USPSTF updated its preeclampsia aspirin recommendation in 2021. The 2021 guideline recommends initiating low-dose aspirin (81 mg/day) at 12 weeks gestation (not after 12 weeks), and expanded the eligible population with updated risk criteria. The question stem says 'after 12 weeks gestation' which may conflict with current guidance to start at or around 12 weeks. Additionally, the 2023 USPSTF reaffirmation and ACOG guidance specify initiation between 12-28 weeks (optimally before 16 weeks), making the 'after 12 weeks' framing potentially misleading.

Q109

Ranitidine (Zantac) was withdrawn from the US market in April 2020 by the FDA due to NDMA contamination concerns. Its inclusion as an answer choice could confuse students into thinking it remains a viable option. Additionally, treatment guidelines for eosinophilic esophagitis have evolved; current AGA/ACG guidelines emphasize PPIs as first-line (not just budesonide), and dupilumab (biologics) are now FDA-approved for EoE as of 2022, potentially changing the landscape of 'correct' answers.

Q110

Current VA/DoD and APA PTSD guidelines (updated 2023) list SSRIs (sertraline, paroxetine) and SNRIs (venlafaxine) as first-line, but the 2023 VA/DoD Clinical Practice Guideline strongly recommends trauma-focused psychotherapies (CPT, PE, EMDR) over pharmacotherapy as first-line treatment, and venlafaxine has a conditional rather than strong recommendation. However, among pharmacologic options venlafaxine remains acceptable. More concerning is that prazosin, previously recommended for PTSD nightmares, has fallen out of favor. The question is borderline but the framing around 'has been receiving' some unspecified treatment and the answer choices do not include SSRIs (first-line) which could mislead students about the pharmacotherapy hierarchy.

Q114

Ranitidine (Zantac) was withdrawn from the US market in 2020 due to NDMA contamination. Including it as a distractor (rather than the correct answer) is fine in principle, but the question lists it as a current medication in a patient's regimen, which could normalize a drug no longer available. More importantly, students should know ranitidine is off the market entirely.

Q115

Current evidence and USPSTF/SVS guidelines emphasize that smoking history (ever-smoker, 100+ cigarettes lifetime) combined with age 65-75 and male sex are co-equal major risk factors for AAA screening. Some current sources list male sex as the strongest single demographic risk factor, and the framing of 'duration of smoking' vs. 'ever smoking' may conflict with current screening criteria thresholds. The answer is debatable and could mislead students about USPSTF AAA screening recommendations.

Q117

Pulmonary nodule management guidelines (Fleischner Society 2017 and subsequent updates) characterize ground-glass/nonsolid nodules as generally lower risk and often requiring longer follow-up intervals rather than being flagged as the highest-risk feature. Solid nodules with irregular borders or spiculation are now more strongly emphasized as the highest-risk morphology. The answer selecting 'nonsolid ground glass appearance' as the most concerning feature could mislead students about current Fleischner Society risk stratification.

Q120

Current guidelines for adolescent knee pain with the described presentation (gradual onset, activity-related, no trauma, soccer player, age 13) are consistent with Osgood-Schlatter or patellofemoral syndrome, for which no imaging is initially recommended. However, in a 13-year-old female with progressive knee pain, current pediatric orthopedic and ACR guidelines recommend radiography to rule out osteosarcoma or other bony pathology before concluding no imaging is needed. The 'no imaging' answer may be misleading given current practice patterns for adolescent bone pain.

Q123

CAP treatment guidelines have evolved. The 2019 ATS/IDSA CAP guidelines and subsequent updates emphasize that for hospitalized non-ICU patients, beta-lactam plus macrolide remains acceptable, but azithromycin resistance concerns and the 2021 IDSA/ATS update recommend considering respiratory fluoroquinolone monotherapy as an alternative first-line. More importantly, for a nursing home resident (healthcare-associated exposure), current guidelines may favor different coverage. The question context (nursing home, COPD, multiple comorbidities) raises questions about whether standard CAP regimen vs. HAP/HCAP coverage applies, and HCAP as a category has been largely abandoned in current guidelines.

Q124

Current 2022 AHA/ACC heart failure guidelines now recommend SGLT2 inhibitors (e.g., dapagliflozin, empagliflozin) as foundational therapy for HFrEF alongside the four pillars of care. Sacubitril/valsartan is now preferred over ACE inhibitors as first-line RAAS therapy for HFrEF. The framing of this question and the correct answer (furosemide for symptom relief) may still be technically correct for symptomatic volume overload, but the question setup and answer choices could mislead students about current guideline-directed medical therapy priorities for HFrEF, particularly given the prominence now given to ARNI and SGLT2i as preferred agents over ACE inhibitors/ARBs.

Q126

USPSTF updated its Hepatitis C screening recommendation in 2020 to recommend screening for all adults aged 18-79 years (grade B), which would apply to this 38-year-old patient. If hepatitis C screening now applies to this patient, it would be a correct answer competing with intimate partner violence screening, potentially making the question ambiguous or misleading for students learning current USPSTF recommendations.

Q135

Choice D references ranitidine (Zantac), which was withdrawn from the US market in 2020 by the FDA due to NDMA contamination concerns. A question listing ranitidine as an active therapeutic option could confuse students about currently available treatments, and the distractor is no longer valid.

Q150

The ISCHEMIA trial (2019) and subsequent guidelines have changed the management of stable ischemic heart disease. Additionally, the question implies amlodipine is the preferred add-on for refractory angina in a post-MI patient already on optimal medical therapy, but current ACC/AHA guidelines (2023 Chronic Coronary Disease guideline) emphasize a broader approach including ranolazine, long-acting nitrates, and beta-blockers as first-line, and the correct answer choice may not reflect current nuanced guidance on add-on antianginal therapy selection.

Q156

Buprenorphine initiation guidance has changed significantly. The 2023 SAMHSA guidelines and emerging low-threshold/low-dose buprenorphine (LDLD) protocols now support initiation without requiring the patient to be in moderate withdrawal (COWS score-based). 'Low-dose' or 'micro-dosing' induction protocols allow initiation in patients not in withdrawal, and the traditional requirement to wait for mild-to-moderate withdrawal before initiating buprenorphine (to avoid precipitated withdrawal) is no longer the only accepted approach. Teaching that buprenorphine 'should be initiated when in mild to moderate withdrawal' may be outdated given current micro-induction protocols now endorsed in clinical practice.

Q159

Hydrochlorothiazide as a medication to discontinue in an 85-year-old may reflect older Beers Criteria thinking, but more critically, current 2023 ACC/AHA hypertension guidelines and geriatric deprescribing frameworks have evolved. However, the more significant concern is that thiazide diuretics are still generally acceptable in elderly patients for hypertension, and the question stem is ambiguous about why HCTZ is the 'most appropriate to discontinue.' If the answer key logic is based on fall risk or electrolyte concerns in the elderly, current guidelines (2023 AGS Beers Criteria) still list thiazides with caution but do not clearly rank them above other listed medications for deprescribing priority. This could confuse students about current deprescribing recommendations.

Q160

GOLD 2023/2024 COPD guidelines have significantly revised initial pharmacotherapy recommendations. For stable COPD, the updated GOLD strategy (2023 ABE grouping replacing ABCD) recommends LAMA (tiotropium) as first-line for group B/E patients, which aligns with the answer. However, the current GOLD 2023 guidelines now recommend dual LABA/LAMA as initial therapy for many symptomatic patients (Group B with significant symptoms/high risk), and the question framing with FEV1 75% predicted and symptom assessment may no longer clearly point to LAMA monotherapy as the unambiguous first-line choice. The revised GOLD grouping (removing the C group and merging C/D into E) changes how this clinical scenario would be categorized and managed today.

Q164

Current guidelines (AAOS, AAFP) for mild-to-moderate carpal tunnel syndrome favor conservative first-line treatment with wrist splinting. Corticosteroid injection is effective but is generally considered after conservative measures fail, not as the preferred initial treatment when splinting has not yet been tried. A student selecting corticosteroid injection as the best initial treatment over night splints could be misled about first-line management.

Q165

Current ESC/AHA guidelines for acute pericarditis recommend combination therapy with both ibuprofen (or aspirin) AND colchicine as first-line treatment, not ibuprofen monotherapy alone. A student selecting ibuprofen alone as the correct answer would be learning an outdated approach that does not reflect current evidence-based practice, which emphasizes colchicine added to NSAIDs to reduce recurrence risk.

Q169

Pediatric hypertension classification and blood pressure thresholds were significantly updated by the AAP 2017 Clinical Practice Guideline. The question context and answer choices may reflect older staging and management thresholds. Additionally, blood pressure cutoffs and the definition of elevated BP versus stage 1 hypertension in adolescents changed, and the recommended follow-up intervals for elevated readings changed. A student should verify the question aligns with the 2017 AAP guidelines currently in use.

Q170

USPSTF and AAP screening recommendations for developmental dysplasia of the hip (DDH) have been reviewed and updated. The USPSTF (2021) found insufficient evidence to recommend universal screening. Current AAP and Pediatric Orthopaedic Society of North America risk factor criteria for ultrasound screening (breech presentation, family history, female sex) have been refined. The specific risk factors listed in the answer choices may not precisely match current guideline thresholds — notably, whether a male infant with a first-degree relative warrants imaging differs by guideline, and the exact indications for ultrasound vs. clinical monitoring have been updated, potentially making this question misleading.

Q171

USPSTF updated fluoride supplementation recommendations. Current guidelines recommend primary care clinicians prescribe oral fluoride supplementation starting at age 6 months for children whose water supply is fluoride deficient, which matches answer B. However, well water fluoride content must be tested first — blanket supplementation for all well water children (answer A) is not recommended, and the nuance around varnish application intervals has been updated. The AAFP/AAP also now recommend fluoride varnish application starting at primary tooth eruption (not waiting until all primary teeth come in). The correct answer B remains consistent, but the question context around varnish timing in distractor D reflects outdated AAP schedules (current AAP recommends varnish at each preventive visit starting at first tooth eruption, not 'once all primary teeth have come in'). Flag due to potential confusion with updated varnish guidelines.

Q173

GOLD COPD guidelines define airflow obstruction as a post-bronchodilator FEV1/FVC ratio <0.70 (i.e., $<70\%$), which is irreversible. Answer C states 'irreversible FEV1/FVC ratio $<65\%$ ' which uses an incorrect threshold. The correct diagnostic criterion per current GOLD guidelines is a fixed ratio <0.70 . A student learning $<65\%$ as the cutoff would have the wrong value on a current exam.

Q177

Current IDSA Lyme disease guidelines (2020 update) recommend doxycycline 100 mg twice daily (or 200 mg once daily) for 10-14 days for early localized Lyme disease, which is consistent with answer D. However, the 2020 IDSA guidelines also endorse doxycycline 4.4 mg/kg/day in two divided doses for 10 days and clarify treatment duration nuances. More importantly, single-dose doxycycline 200 mg (answer C) is the recommended prophylaxis for tick bites, and this question context (symptomatic patient with confirmed exposure) makes the correct answer D still valid. However, the duration in the guidelines has shifted to explicitly include 10-14 days rather than just 10 days in some presentations. Flag as borderline due to 2020 IDSA guideline revision that may affect interpretation.

Q182

Current guidelines and evidence have evolved regarding fluoroquinolone use. The FDA has issued multiple safety communications restricting fluoroquinolone use, and the specific association between fluoroquinolones (particularly levofloxacin) and aortic aneurysm rupture has led to updated FDA black box warnings in 2018 and further restrictions. Additionally, current CAP guidelines (ATS/IDSA 2019) have updated preferred antibiotic regimens for hospitalized CAP patients, and fluoroquinolone use patterns have changed significantly. Students should know the current first-line CAP regimens which have been updated.

Q187

Ranitidine (Zantac) was withdrawn from the market in 2020 due to NDMA contamination concerns. While metoclopramide remains the correct answer, the inclusion of ranitidine as a distractor is problematic and could confuse students. Additionally, current guidelines note significant concerns about metoclopramide's long-term use (black box warning for tardive dyskinesia), and newer agents/approaches may be preferred; the question context may mislead students about the current treatment landscape for diabetic gastroparesis.

Q193

Epididymitis treatment guidelines have been updated. Current CDC 2021 STI guidelines recommend ceftriaxone 500mg IM single dose plus doxycycline 100mg BID x 10 days for sexually transmitted epididymitis. The question may still point to the right combination, but the ceftriaxone dose has changed significantly (from 250mg to 500mg), and students could be misled about dosing specifics that are tested on current boards.

Q200

Current IDSA/AAP guidelines for acute bacterial rhinosinusitis recommend amoxicillin-clavulanate (not doxycycline) as first-line therapy. High-dose amoxicillin-clavulanate is preferred for adults, especially those with risk factors. Doxycycline is considered an alternative for penicillin-allergic patients, not the primary first-line agent. Marking doxycycline as correct could mislead students about current first-line sinusitis management.

2020 Examination — 45 questions removed**Q1**

Hypertension treatment guidelines have evolved. The 2017 ACC/AHA guidelines (widely adopted) lowered the threshold for Stage 2 hypertension and emphasize thiazide-type diuretics, ACE inhibitors, ARBs, or CCBs as first-line agents. For Asian patients specifically, ACE inhibitors carry higher risk of cough and some guidelines favor ARBs or CCBs as preferred first-line agents in this population. The combination product as first-line for a patient with no comorbidities is debatable and may not reflect current nuanced guideline recommendations, potentially misleading students about preferred initial monotherapy vs. combination and ethnic considerations.

Q2

CDC influenza chemoprophylaxis guidelines for long-term care facilities have been updated. Current CDC guidance recommends antiviral chemoprophylaxis for ALL residents of a long-term care facility (not just those on the same unit) when an influenza outbreak is confirmed, regardless of vaccination status. Limiting prophylaxis to only the same unit may no longer reflect current best practice and could mislead students.

Q5

Cervical cancer screening guidelines have changed. The 2020 USPSTF guidelines and updated ACS guidelines now recommend that HPV co-testing (or primary HPV testing) begin at age 25, not age 30. The correct answer of age 30 reflects older ACS guidance; current ACS guidelines (2020 update) actually recommend primary HPV testing starting at age 25, and USPSTF recommends co-testing or cytology alone starting at age 21-30 with co-testing from 30-65. A student today could be misled about the correct starting age for co-testing.

Q11

Current acne guidelines (AAD 2024) recommend against topical antibiotic monotherapy and emphasize that topical antibiotics should be combined with benzoyl peroxide to reduce antibiotic resistance. The correct answer of adding clindamycin alone without benzoyl peroxide is no longer consistent with current best practice, which would recommend a fixed-dose combination (e.g., clindamycin/benzoyl peroxide) rather than clindamycin 1% gel alone.

Q14

The USPSTF updated its fall prevention recommendations in 2024. The 2024 recommendation found insufficient evidence for exercise interventions to prevent falls in community-dwelling adults 65+ who are not at increased fall risk, and the specific recommendations regarding which interventions have the 'strongest evidence' have been revised. Vitamin D supplementation recommendations also changed. A student could be misled about current USPSTF fall prevention guidance.

Q27

ScoliScore (option A) has largely fallen out of favor and is no longer recommended in current AAP/AAOS/SRS guidelines for scoliosis workup. While the correct answer (orthopedist referral) is still appropriate for a 32° Cobb angle, the inclusion of ScoliScore as a distractor could mislead students into thinking it remains a relevant clinical tool, and the framing of the question may not reflect current screening and management thresholds which have been updated in recent pediatric orthopedic guidelines.

Q28

Current guidelines (AAO-HNS, AAFP) recommend cerumenolytic agents (water, saline, oil-based drops, or commercial softeners) as first-line treatment for cerumen impaction. Peroxide-based drops (carbamide peroxide) are acceptable, but framing them as the preferred answer over olive oil or other softeners may be misleading. More importantly, for a patient with dementia, manual removal under direct visualization by a trained provider is often recommended when irrigation is contraindicated or risky — the answer hierarchy among these choices may not align with current best-practice algorithms for vulnerable/demented elderly patients. This question could reinforce incorrect prioritization of treatments.

Q34

USPSTF updated aspirin recommendations significantly. In 2022, USPSTF now recommends against initiating aspirin for primary prevention in adults 60 and older (Grade D), and recommends against routine aspirin for primary prevention in adults 40-59 with <10% CVD risk. While stopping aspirin may still be correct, the nuance and reasoning have changed substantially with the 2022 USPSTF update, and students need to know the current grade D recommendation for age 60+ rather than the older framework.

Q38

USPSTF updated depression screening recommendations in 2023. The current recommendation screens adults including pregnant and postpartum persons, but the 2023 update added nuance and the recommendation for adolescents (12-18) is separate. More importantly, the USPSTF 2023 update for depression in adults reaffirmed screening for all adults, but the question may be outdated regarding whether it adequately reflects the current scope including pregnant/postpartum populations and the updated evidence base. Additionally, the 2023 USPSTF update on screening for depression, anxiety, and other conditions should be verified against current practice.

Q43

Bisphosphonate holiday recommendations have been refined. For a patient with T-score of -2.7 and high fracture risk, current guidelines (ASBMR, Endocrine Society) suggest that after 5 years of oral bisphosphonate therapy, a drug holiday may be considered for lower-risk patients, but the framing of 'continue 3 more years' (to reach 5 total) vs current nuanced reassessment guidance at 3-5 years could mislead students. More importantly, the FRAX score of 5% hip fracture probability is actually above the intervention threshold, and current guidelines emphasize reassessment at 3-5 years with individualized decisions — the answer as stated may not align with the most current 2023-2024 ASBMR guidelines on bisphosphonate duration and holiday timing.

Q46

While mechanical heart valves remain a contraindication to DOACs (warfarin preferred), the question also lists 'mild mitral stenosis' as a distractor. Current 2023 ACC/AHA guidelines updated recommendations for mitral stenosis and anticoagulation — moderate-to-severe rheumatic mitral stenosis now has specific DOAC vs warfarin guidance that has evolved, and the framing of this question may not reflect current nuanced guidance on valvular AF and anticoagulant selection.

Q55

The preferred oral iron for non-dialysis-dependent CKD has evolved. Current KDIGO and nephrology guidelines do not clearly single out ferric citrate as superior to standard ferrous salts for non-dialysis CKD patients; ferric citrate is FDA-approved primarily for dialysis-dependent CKD for hyperphosphatemia management with a secondary benefit on iron. Selecting ferric citrate as 'most effective' over standard ferrous preparations for non-dialysis CKD is not well-supported by current guidelines and could teach students an incorrect hierarchy of iron supplementation.

Q59

Current guidelines and practice for evaluation of palpitations occurring daily favor extended cardiac monitoring beyond 24-hour Holter. Current ACC/AHA and Heart Rhythm Society guidelines recommend longer-duration monitoring (e.g., 2-week or 30-day event monitor or patch monitor) when symptoms are frequent but not captured on shorter monitoring. A student today might be taught that a 30-day event monitor or extended Holter (up to 14 days) is more appropriate for daily palpitations, making the 24-hour Holter the suboptimal first choice in current practice.

Q60

GOLD 2023/2024 guidelines have updated management of COPD. For a patient with GOLD GRADE 1 (mild) COPD who is asymptomatic (no dyspnea, cough, or sputum), 'no treatment' may still be defensible, but current GOLD guidelines recommend offering pharmacotherapy (bronchodilator) even in low-symptom patients with confirmed airflow obstruction. Additionally, current USPSTF (2022) and GOLD guidelines place greater emphasis on treating even mild COPD with bronchodilators when spirometry confirms obstruction. This question could mislead students about current treatment thresholds.

Q62

Wegener's granulomatosis (choice E) has been renamed granulomatosis with polyangiitis (GPA) in current nosology. While the correct answer (temporal arteritis, now called giant cell arteritis) remains accurate, the outdated terminology in the distractors could reinforce incorrect nomenclature for students learning current classification.

Q63

Current ASCO/NCCN guidelines for colorectal cancer surveillance recommend CEA monitoring AND periodic CT imaging and colonoscopy. The framing suggesting CEA levels only as the correct answer for routine follow-up blood tests may be misleading, and guidelines have been updated to clarify the role of CEA alongside other monitoring modalities. Students should not be taught that CEA alone constitutes adequate surveillance planning.

Q69

Current guidelines (AAP/ACOG) define primary amenorrhea as absence of menarche by age 15 in the presence of secondary sexual characteristics, which matches the answer. However, some updated guidelines now suggest evaluation should begin at age 15 but the threshold for concern/workup has nuance with updated 2023 AAP guidance on puberty and menstrual health. This is borderline but flagging due to updated pediatric endocrine society guidance that may shift evaluation thresholds.

Q71

For hypertension in a diabetic patient, current ADA and ACC/AHA guidelines recommend ACE inhibitors or ARBs as first-line (especially with diabetes and hypertension for renal protection), not calcium channel blockers. The question's correct answer of CCB over ARB may mislead students about current first-line therapy for hypertension in diabetes. Additionally, 2017 ACC/AHA guidelines lowered BP thresholds and updated preferred agents.

Q75

ACC/AHA perioperative guidelines have been updated. Current guidelines (2014 ACC/AHA, reaffirmed/updated) do not recommend routine preoperative EKG or testing for low-risk procedures like cataract surgery regardless of age. While 'no testing' may still be correct, the context of prediabetes and erectile dysfunction (possible cardiovascular risk factors) and whether current guidelines still unambiguously support no testing for a 70-year-old warrants review. However, 'no testing' for cataract surgery remains current — flagging as borderline given age and comorbidities may prompt reconsideration.

Q85

USPSTF HIV screening recommendations have evolved. Current 2019 USPSTF guidelines recommend HIV screening for all adults aged 15-65 regardless of risk factors. For a 45-year-old, HIV screening would be recommended as universal screening, not risk-based. Additionally, USPSTF now recommends chlamydia/gonorrhea screening for sexually active women at increased risk - the framing of this question around a woman who has sex only with women may mislead students about current universal HIV screening recommendations versus targeted STI screening. The question context could reinforce outdated risk-stratified rather than universal HIV screening thinking.

Q87

Current ADA and ACOG guidelines for postpartum diabetes management in women with type 2 diabetes have nuanced recommendations. Metformin is considered acceptable postpartum and during breastfeeding, but current guidelines also support GLP-1 agonists postpartum with caution during breastfeeding. More importantly, current guidelines emphasize that many women with gestational or type 2 diabetes may not need immediate reinitiation of all prior agents and the framing around liraglutide safety postpartum/breastfeeding has been updated - liraglutide is generally avoided in breastfeeding per current labeling, but the categorical answer here may not reflect current nuanced postpartum glycemic management guidance.

Q90

Current ADA 2024 guidelines now emphasize SGLT2 inhibitors and finerenone as preferred agents for reducing progression of diabetic kidney disease, alongside ACE inhibitors/ARBs. While ARBs like valsartan remain recommended, the question as framed may mislead students into thinking ARBs are the primary/only answer for renoprotection in diabetic kidney disease, when current guidelines now prioritize SGLT2 inhibitors (e.g., empagliflozin, dapagliflozin) and finerenone as first-line renoprotective agents in CKD with diabetes, potentially making this answer incomplete or misleading relative to current best practice.

Q105

Current evidence and guidelines for displaced midshaft clavicle fractures have evolved. The question stem mentions a 'painful palpable lump' suggesting possible displacement, and the correct answer is surgical fixation evaluation. However, without specifying the degree of displacement/shortening in the stem, this question may mislead students. More importantly, current orthopedic and family medicine guidelines (AAOS, AAFP) still support non-operative management (sling) for most midshaft clavicle fractures including many displaced ones, reserving surgical referral for specific indications (>2cm shortening, open fracture, neurovascular compromise, floating shoulder). The question as written may teach overly aggressive surgical referral without adequate clinical criteria.

Q109

Pneumococcal vaccine recommendations have changed significantly. The question likely intended to test knowledge of Shingrix (still correct for zoster), but the pneumococcal options (PCV13 vs PPSV23) reflect outdated ACIP guidance. Current 2023-2024 ACIP recommendations recommend PCV15 or PCV20 for adults ≥65, replacing the old PCV13/PPSV23 sequential strategy. A student seeing PCV13 (Pneumovax 13) as the only conjugate option would be taught outdated vaccine choices. Additionally, at age 50 with no immunocompromising conditions, Shingrix is now recommended starting at age 50, which remains correct, but the pneumococcal distractor options could reinforce outdated thinking.

Q110

For generalized anxiety disorder (GAD), current first-line pharmacotherapy guidelines (APA, AAFP) recommend SSRIs or SNRIs. While duloxetine (SNRI) remains an acceptable option, SSRIs (e.g., escitalopram, sertraline) are generally listed as first-line before SNRIs in most current guidelines. The absence of an SSRI option and positioning duloxetine as the correct answer over buspirone (also guideline-supported first-line for GAD per some guidelines) could mislead students about current treatment hierarchy. Buspirone is also considered a first-line option for GAD in current practice, making this answer potentially ambiguous by current standards.

Q114

The 17-year-old male scenario may be outdated. Current ACIP/CDC recommendations (2019 onward) extended the catch-up HPV vaccine recommendation through age 26 for all, and for ages 27-45 shared decision-making applies. More importantly, the two- vs three-dose distinction: the 17-year-old qualifies for catch-up but if he initiates before age 15 he gets 2 doses; starting at 15 or older he gets 3 doses. At age 17, 3 doses is correct. The 11-year-old gets 2 doses. Answer D may still be correct, but students should verify current ACIP schedules as nuances have been updated and the question context about the male's sexual history (no evidence of prior infection) and the specific age cutoffs for 2 vs 3 doses should be confirmed against the most current 2023-2024 ACIP schedule.

Q118

This question asks which medication is most likely causing hepatic steatosis/NAFLD findings. The answer is ibuprofen (NSAID). However, current guidelines (2023 AGA/AASLD) have updated NAFLD nomenclature to MASLD/MASH, and more importantly, current evidence and guidelines now emphasize that metformin and statins are generally safe or even beneficial in NAFLD/MASLD. The framing of the question may mislead students about the relative hepatotoxicity of these agents. Additionally, current guidelines support continuing statins in NAFLD patients, and the implication that metformin or atorvastatin are more likely culprits than NSAIDs could confuse students given updated guidance on MASLD management.

Q122

Current AAP and CDC guidance acknowledges widespread permethrin resistance in head lice. Ivermectin 0.5% lotion (Sklice) and spinosad are now considered preferred first-line agents by many current guidelines due to permethrin resistance, making permethrin a potentially misleading 'correct' answer for students today.

Q123

Current guidelines for hypertensive urgency (no end-organ damage) recommend restarting/adjusting oral home medications and close outpatient follow-up, NOT administering oral labetalol repeatedly in urgent care. The correct answer B reflects an overly aggressive approach that is no longer considered best practice for asymptomatic hypertensive urgency.

Q124

USPSTF updated its breast cancer screening guidelines in 2024. The current recommendation is biennial screening mammography for ALL women starting at age 40 (not 50), through age 74. Answer D (ages 50-75) is now incorrect and would teach students outdated guidance.

Q135

IE prophylaxis guidelines have evolved. The 2021 AHA/ACC updated guidance on transcatheter valve replacements (TAVR) and IE prophylaxis. Current AHA guidelines (2021 valvular heart disease) indicate that patients with transcatheter-implanted prosthetic valves should receive IE prophylaxis, but the question should be verified against the most current 2021 ACC/AHA valve guidelines. Additionally, rheumatic mitral valve disease (choice D) and the other distractors' classification may have shifted. The transcatheter aortic valve replacement answer aligns with 2021 AHA/ACC guidelines making this worth flagging as the specific categorization of high-risk lesions was updated in 2021.

Q140

The USPSTF hepatitis C screening recommendations changed in 2020 to recommend screening for all adults aged 18-79, no longer just birth cohort 1945-1965. The question references a patient 'born in 1954' framing that reflects the older birth-cohort screening paradigm. More importantly, current HCV testing algorithms (CDC 2020, AASLD) recommend a reflex HCV RNA test when anti-HCV is positive rather than a separate 'qualitative HCV RNA test' order, and the terminology/workflow has been updated. The answer choice B may still be directionally correct but the framing around birth cohort screening is outdated.

Q148

AAP/AAPD guidelines recommend a smear of fluoride toothpaste (NOT 'low-fluoride') for children under 3, as soon as the first tooth erupts. The distinction 'low-fluoride' is outdated; current guidance specifies a rice-grain/smear amount of regular fluoride toothpaste. Additionally, current USPSTF and AAP guidelines recommend that primary care clinicians apply fluoride varnish to primary teeth starting when teeth erupt, which is not reflected in the answer choices.

Q153

The USPSTF updated its vision screening recommendation in 2017 and reaffirmed considerations since. The current USPSTF recommendation (2017, still active) recommends vision screening for all children at least once between ages 3 and 5 years to detect amblyopia or its risk factors — so answer B may still be technically correct, but the question wording and answer choices should be verified against the most current USPSTF statement to ensure no updates have occurred since 2020 that would change the recommended age range or modality.

Q159

IDSA/ACG C. difficile guidelines were updated in 2021. For a non-severe initial CDI episode, vancomycin 125 mg four times daily for 10 days OR fidaxomicin 200 mg twice daily for 10 days are both recommended first-line options, with fidaxomicin now preferred over vancomycin in many guidelines due to lower recurrence rates. Metronidazole is no longer recommended as first-line therapy. The correct answer of fidaxomicin is consistent with current guidelines, but the framing of this question (post-hospitalization, diabetic patient) and the inclusion of metronidazole as a plausible option could reinforce outdated thinking that metronidazole remains a viable first-line choice. The question may mislead students about the current equivalence or preference between fidaxomicin and vancomycin.

Q164

AAP 2017 guidelines (still current) define target BP for children with hypertension as <90th percentile for age, sex, and height for those under 13, but the 2017 AAP Clinical Practice Guideline also introduced new normative tables. However, more importantly, for an obese 10-year-old with secondary/primary hypertension, current guidelines (AAP 2017, reaffirmed) target <90th percentile which may still be correct, but the question warrants review as some interpretations now align pediatric targets closer to adult equivalents (<130/80) for adolescents. Flag for review as the AAP 2017 guideline nuances around obesity-related hypertension targets have evolved and may cause confusion.

Q168

ADA and ACC/AHA guidelines for statin initiation in diabetic patients have evolved. Current ADA 2024 Standards of Care recommend moderate-intensity statin for all patients with diabetes aged 40-75 regardless of ASCVD risk, and for patients under 40 with additional risk factors, statin therapy may be considered. The answer 'statin therapy at 40 years of age' may conflict with current guidance which in some framings suggests earlier initiation based on risk factors. Additionally, ADA 2024 emphasizes that patients with diabetes and LDL ≥ 70 mg/dL plus risk factors may warrant statin before age 40. This could mislead students into thinking statins should be withheld until age 40 unconditionally.

Q169

Current guidelines and literature from ADA, ISPAD, and diabetes organizations no longer categorically state type 1 diabetes patients must avoid Ramadan fasting. The 2021 and later consensus recommendations indicate that with proper education, glucose monitoring, and insulin adjustment (using insulin analogs like glargine with dose reduction), fasting during Ramadan can be done safely in selected, motivated type 1 diabetic patients under close medical supervision. Categorically advising avoidance is considered outdated paternalistic guidance that contradicts current shared decision-making approaches.

Q179

AAP bronchiolitis guidelines updated: oxygen saturation threshold for intervention has been debated, with current guidance suggesting >90% remains acceptable but deep nasal suctioning recommendation context has evolved. More critically, the oxygen saturation threshold language should be reviewed against current AAP 2023 guidance which maintains >90% but with nuanced context around routine suctioning practices.

Q180

USPSTF screening recommendations have been updated. The correct answer of intimate partner violence screening remains current (USPSTF B recommendation for women of reproductive age), but skin cancer counseling recommendations were updated by USPSTF in 2023 (I statement for adults >6 years), and vitamin D screening recommendations have also been updated. The question context and correct answer appear stable, but other distractors may reflect changed guideline statuses that could confuse students.

Q182

The recommendation to avoid seeds, nuts, and popcorn in diverticular disease has been officially abandoned by current guidelines (ACG, AGA). The correct answer 'rest and clear liquids' for uncomplicated diverticulitis is also evolving — current guidelines (ACG 2020) suggest that antibiotics may be selectively omitted in uncomplicated diverticulitis, and dietary restrictions are no longer routinely recommended. A student may be misled about the management approach for uncomplicated diverticulitis.

Q184

Current CDC, AHA, and AAFP guidelines strongly discourage opioids as preferred/continued therapy for chronic non-cancer pain, especially at the dose described. The 2022 CDC Clinical Practice Guideline for Prescribing Opioids recommends non-opioid therapies as preferred first-line for chronic pain. Selecting oxycodone as the correct ongoing treatment could reinforce outdated prescribing practices that are now considered harmful.

Q190

For a febrile seizure workup in a fully immunized infant, current AAP guidelines (2011, reaffirmed and still current) do not recommend routine chest radiograph unless respiratory symptoms are present. However, the clinical stem suggests a febrile seizure and the correct answer of chest X-ray could be misleading. More critically, if the question is about non-accidental trauma evaluation, the workup has been updated. The answer directing toward chest radiograph as the primary next step for a febrile seizure without respiratory findings could mislead students away from current evidence-based evaluation.

Q196

USPSTF (2021) concluded there is insufficient evidence (I statement) to recommend screening for vitamin D deficiency in asymptomatic adults. The correct answer pointing to CKD patients (GFR 28) reflects older thinking where population screening was more broadly endorsed. Current USPSTF does not recommend routine screening for vitamin D deficiency in asymptomatic adults; CKD management guidelines (KDIGO) do address vitamin D but the framing of this question as a 'screening' question could mislead students about current USPSTF recommendations.

Q199

Current ACR guidelines (2022) for glucocorticoid-induced osteoporosis recommend that patients starting long-term glucocorticoids (≥ 2.5 mg/day prednisone for ≥ 3 months) who are at moderate-to-high fracture risk should receive pharmacologic treatment (bisphosphonate). For a 63-year-old female starting 20 mg prednisone for anticipated months of treatment, current guidelines recommend initiating a bisphosphonate (e.g., alendronate) often without waiting for DXA/FRAX, especially in postmenopausal women on high-dose steroids. The answer of obtaining FRAX/DXA first before treating may underemphasize the 2022 ACR recommendations that favor earlier pharmacologic intervention in this population.

2021 Examination — 36 questions removed**Q4**

Buprenorphine induction protocols have evolved significantly. Low-dose (microdosing/Bernese) induction is now widely used and does not require waiting for withdrawal. Additionally, for patients on short-acting opioids, the standard wait time guidance of 8-12 hours may still apply for traditional induction, but the emergence of buprenorphine extended-release injectable (Sublocade) and updated SAMHSA guidelines emphasizing low-dose induction strategies mean this question's framing as a single correct answer could mislead students about current practice options.

Q10

Current IDSA and AAP guidelines recommend oral dicloxacillin or cephalexin as first-line for bullous impetigo when systemic therapy is needed, as bullous impetigo is caused by *S. aureus* toxin-producing strains. TMP-SMX is appropriate for MRSA coverage but is not reliably recommended over beta-lactams for routine bullous impetigo treatment in children. The correct answer depends on local MRSA prevalence and the question does not specify MRSA concern; current guidelines favor beta-lactams (cephalexin) as first-line for bullous impetigo rather than TMP-SMX, making this answer potentially misleading for current practice.

Q13

Statin contraindication in pregnancy guidance has evolved. While statins remain contraindicated in pregnancy, the FDA removed the blanket Pregnancy Category X designation in 2021 and updated labeling acknowledges that some high-risk patients may continue statins. More importantly, the framing of 'absolute contraindication' vs. individualized risk discussion has shifted. Additionally, myositis with CK $>5\times$ ULN (choice D) is now considered a reason to hold/discontinue statins per current ACC/AHA guidance, which could confuse students about what constitutes a true contraindication vs. a management trigger — the question may teach incorrect reasoning about the other distractors.

Q17

The ADA diagnostic criteria for diabetes include fasting plasma glucose ≥ 126 mg/dL, not 130 mg/dL. A fasting plasma glucose of 130 mg/dL does meet criteria, but choice A (HbA1c of 6.4%) is prediabetes, not diabetes (diabetes threshold is $\geq 6.5\%$), and choice B (2-hour glucose of 190 mg/dL) does not meet the diabetes threshold of ≥ 200 mg/dL. However, the question asks which value confirms diabetes — 130 mg/dL fasting does exceed the 126 mg/dL cutoff so D is correct, but the 2-hour OGTT threshold for diabetes is ≥ 200 mg/dL (190 is prediabetes range), making this question potentially confusing but not technically wrong. Still, students should note the exact ADA cutoffs: FPG ≥ 126 , 2hr ≥ 200 , A1c $\geq 6.5\%$, which this question correctly reflects — flagging as borderline due to potential confusion around exact thresholds.

Q25

Ipratropium (Atrovent) as the 'best treatment' for symptomatic relief of persistent cough in acute upper respiratory infection is not well-supported by current guidelines. Current AAFP/CHEST guidelines do not recommend ipratropium as first-line for post-URI cough; evidence for its benefit is limited and largely superseded by recommendations focusing on honey (in appropriate ages), dextromethorphan, or watchful waiting. Selecting ipratropium could reinforce a treatment approach not consistent with current evidence-based practice.

Q28

The question about testosterone therapy in a 72-year-old female with fatigue and depression having 'no proven benefit' may be outdated. Current evidence and guidelines have evolved regarding hormone therapy considerations in older women, and the framing of the question may mislead students about current assessment of HRT or testosterone use in this population. Additionally, the clinical scenario (fatigue, weight gain, depression) without TSH or other workup being mentioned raises concern the question is testing an outdated management algorithm.

Q40

The correct answer is listed as bupropion (Wellbutrin XL), but current evidence and guidelines (AAFP/APA) support light therapy as first-line treatment for seasonal affective disorder (SAD). Bupropion XL is FDA-approved for prevention of SAD but is generally considered alongside or after light therapy, not clearly superior to it as the single best first-line option. A student could be misled into thinking bupropion is definitively preferred over light therapy, which is not supported by current best practice.

Q55

Vitamin D supplementation recommendations have evolved. The USPSTF (2021) found insufficient evidence to recommend vitamin D supplementation for cancer, cardiovascular disease, or diabetes prevention in asymptomatic adults. However, the blanket statement that it 'is not recommended because she is asymptomatic' may conflict with current nuanced guidance, and the USPSTF 2023 update on falls prevention found that vitamin D supplementation for community-dwelling adults ≥ 60 to prevent falls may actually cause harm. The correct answer as stated could mislead students about current fracture/falls prevention recommendations which have been updated.

Q64

The USPSTF reaffirmed its 2006 statement on lead screening in 2019 (I statement — insufficient evidence) for primary care screening in asymptomatic children ages 1-5 without known risk factors. However, students should be aware this remains an area of ongoing policy discussion and some state/local guidelines differ. The answer (I statement) remains technically correct per USPSTF, but the question should be verified against the most current 2024 USPSTF update to confirm no revision has occurred.

Q65

SGLT2 inhibitors like dapagliflozin are associated with Fournier's gangrene (necrotizing fasciitis of the genitoperineum), not a general infection/cellulitis scenario. However, the key issue is that the question appears to ask which medication should be held/discontinued in the setting of acute serious illness. Current 2024 ADA guidelines and FDA labeling recommend holding SGLT2 inhibitors perioperatively and during acute serious illness due to risk of euglycemic DKA. While dapagliflozin is still the answer here (hold the SGLT2i), the clinical framing and answer rationale may reflect the Fournier's gangrene FDA warning context which has evolved — flag for review to ensure the vignette and answer align with current 2024 guidance on sick-day rules and SGLT2i discontinuation.

Q74

Pediatric perioperative fluid management guidelines have evolved. While isotonic (0.9% NaCl) fluids are now preferred over hypotonic solutions in pediatric surgical patients per updated AAP/anesthesia guidelines, the specific recommended formulation for pediatric maintenance fluids has been updated. Current guidelines (post-2018 AAP policy statement) recommend isotonic fluids for most hospitalized children, which does align with answer C, but the question context and specific dextrose concentration recommendations for postoperative pediatric patients have been further refined and may not be fully consistent with this framing.

Q76

COVID-19 isolation and quarantine guidance has changed significantly since 2021. CDC updated recommendations in 2021-2022 shortened isolation periods and eliminated the 14-day quarantine recommendation. The '5 days' answer for quarantine after exposure reflects a specific CDC guidance era that has since been further revised and eventually largely rescinded. Current guidance (2024) no longer recommends a specific quarantine period for exposed asymptomatic individuals, making this question potentially misleading.

Q77

The 2023 ACC/AHA and ADA guidelines have evolved regarding statin use in diabetes. Current ADA Standards of Care (2024) recommend moderate-intensity statin for patients with diabetes aged 40-75 without additional risk factors and LDL <190, but also emphasize that the choice depends on cardiovascular risk calculation. More importantly, current guidelines increasingly support use of SGLT2 inhibitors or GLP-1 agonists as add-on therapy for cardiovascular and renal benefit even when A1c is at goal. While moderate-intensity statin remains a reasonable answer, the framing of the question and the role of SGLT2 inhibitors for CV risk reduction in diabetes has shifted enough that the question may reinforce outdated risk stratification thinking. Additionally, 2023 ACC guidelines on lipid management updated statin intensity recommendations based on risk scoring.

Q84

Asthma management guidelines (GINA 2022+) have significantly updated step therapy. For mild persistent asthma (SABA use ~twice weekly), current GINA preference is as-needed ICS-formoterol (not ICS/LABA as a class broadly), and NAEPP/GINA updates have shifted preferred therapy. The answer 'ICS/LABA as needed' may be partially current but the framing and specific recommendation reflect older GINA stepwise logic. Current GINA 2023 preferred reliever is as-needed low-dose ICS-formoterol for mild asthma, which could confuse students about which ICS/LABA combinations are approved for as-needed use vs. maintenance. This question could reinforce imprecise or outdated step-therapy thinking.

Q88

For symptomatic uterine fibroids with heavy menstrual bleeding and desire to preserve uterus, current guidelines (ACOG, 2021-2024) have elevated the levonorgestrel-releasing IUD and also the oral GnRH antagonists (relugolix/elagolix) as effective medical management options. Additionally, a selective progesterone receptor modulator approach and newer GnRH antagonist combinations are now first-line medical options before procedural intervention. Uterine artery embolization as the correct answer over the levonorgestrel IUD (option D) may not reflect current preferred stepwise management for a 42-year-old who may wish to preserve fertility or try medical management first. Current guidelines would favor trying the LNG-IUD or medical therapy before UAE in many cases.

Q95

Car seat/booster seat guidelines have been updated. The AAP revised recommendations removing specific height/weight thresholds and emphasizing that children should use booster seats until the seatbelt fits properly regardless of height milestone. The '127 cm (50 in)' threshold referenced in answer choice C and the overall framing may not align with current AAP guidance, and the correct answer choice B may also be misleading given updated recommendations that focus on manufacturer limits and proper fit rather than allowing free choice between seat types at this age/size.

Q99

Blood pressure diagnostic thresholds and initial management have evolved. The 2017 ACC/AHA guidelines reclassified 130-139/80-89 as Stage 1 hypertension, and a BP of 141/82 in a young patient with no comorbidities may warrant different initial management considerations under current guidelines. Additionally, current AHA/ACC and JNC guidance on when to start pharmacotherapy vs. lifestyle modification vs. ambulatory monitoring for Stage 1-2 hypertension has been updated, and the correct answer may reflect older thinking about confirming hypertension before treatment in what is now clearly classified as Stage 2 hypertension ($\geq 140/90$).

Q105

For postpartum depression (3-day-old infant, symptoms of depression), current USPSTF and ACOG guidelines (updated 2023) support both pharmacotherapy (particularly sertraline, which is compatible with breastfeeding and evidence-based) and CBT as first-line options. The question marks CBT as the single best answer over sertraline, but current guidelines do not clearly prioritize therapy over medication in this setting — both are first-line, and sertraline is specifically recommended as a preferred antidepressant postpartum. A student could be misled into deprioritizing pharmacotherapy inappropriately.

Q106

The question context describes a potential PEP (post-exposure prophylaxis) situation — a patient exposed to HIV last month. The question asks about initiating antiretroviral therapy, with the correct answer listed as 'at the time of diagnosis of HIV infection.' However, current guidelines (CDC, DHHS) strongly recommend offering nPEP (non-occupational post-exposure prophylaxis) immediately (within 72 hours) regardless of confirmed diagnosis, and then transitioning to PrEP if indicated. The framing and answer choices do not include PEP at all, which is now the standard of care in this exact scenario. A student today could be misled into not offering PEP in a time-sensitive exposure situation.

Q108

GOLD 2022+ guidelines updated the spirometric criterion for COPD. The fixed ratio FEV1/FVC <0.70 post-bronchodilator remains in use, but there is ongoing guideline evolution recommending use of the lower limit of normal (LLN) to avoid overdiagnosis in elderly patients. More importantly, GOLD 2023 introduced a new category (Pre-COPD and PRISm) and updated diagnostic frameworks. The fixed 0.70 cutoff is increasingly recognized as potentially misleading, especially in older adults. While many guidelines still cite 0.70, teaching this as the unambiguous single criterion could reinforce outdated thinking given current GOLD updates.

Q117

Current GOLD/GINA-aligned COPD guidelines favor adding a LABA/ICS combination (triple therapy with LAMA+LABA+ICS) or LABA to LAMA rather than LABA alone as the preferred step-up. More importantly, the preferred add-on to a LAMA is now typically a LABA (forming LAMA+LABA dual bronchodilator), but the answer 'Add inhaled salmeterol (Serevent)' as a standalone choice is potentially misleading because current guidelines (GOLD 2023-2024) recommend dual bronchodilator (LAMA+LABA) combination inhalers as preferred step-up over adding a separate LABA, and the question ignores eosinophil count guidance for ICS add-on decisions. The framing and answer choices may reinforce outdated stepwise thinking that does not reflect current GOLD 2024 personalized treatment algorithms.

Q133

Leading causes of mortality by age group are periodically updated by CDC. For certain age groups (e.g., 10-34 year olds), suicide and accidents compete closely, and the ranking can shift. The question stem is truncated so the specific age group is unclear, but if it references adolescents/young adults, current CDC data (2022-2023) may show different leading causes than what was current in 2021, potentially making a different answer correct today.

Q136

JCIH (Joint Committee on Infant Hearing) guidelines recommend that infants who fail newborn hearing screening should have audiologic evaluation completed by 3 months of age. However, the current 2019 JCIH position statement and subsequent updates emphasize diagnosis by 3 months but some guidelines have moved toward earlier completion (by 2-3 months). This is borderline but worth reviewing against the most current JCIH 2019/2024 guidelines to ensure the '3 months' threshold is still current best practice — it appears consistent with current guidelines, so this may not need flagging. Flagging for review due to potential updates.

Q155

The ATS/IDSA CAP guidelines were updated in 2019 and further clarified. The question references these guidelines but current 2024 practice around procalcitonin use for CAP management has evolved, and some updated guidance (including ATS/IDSA 2019 and subsequent IDSA updates) has modified recommendations around procalcitonin-guided therapy and Legionella urine antigen testing indications. The framing of the question around 'non-severe CAP' and the distractor answers may not align cleanly with the most current 2024 guidance, potentially misleading students about current urine antigen and culture recommendations.

Q158

Azithromycin is listed as a distractor option for syphilis treatment, but current 2021 CDC STI guidelines (updated 2021, reinforced 2024) explicitly state azithromycin should NOT be used due to widespread macrolide resistance in *T. pallidum*. While the correct answer (penicillin G benzathine) remains current, the question stem context around exposure period and management may also reflect older CDC guidance on sexual contact window periods. Additionally, current CDC 2021 guidelines note that for penicillin-allergic patients, doxycycline is preferred over azithromycin — the presence of azithromycin as a plausible option could reinforce outdated thinking.

Q165

Current guidelines (AGA and ACG, updated 2021-2023) no longer emphasize dietary fiber as the primary evidence-based intervention for preventing diverticulitis recurrence, as the evidence has weakened. Furthermore, the question implies antibiotics for uncomplicated diverticulitis (choice C) as a distractor, but current guidelines actually recommend against routine antibiotic use for uncomplicated acute diverticulitis — a concept that conflicts with older teaching embedded in this question's framing.

Q167

The USPSTF updated its hepatitis C screening recommendation in 2020 to screen all adults aged 18-79 years, not 'all adults <80 years of age regardless of risk.' The correct answer (E) approximates current guidance but the question stem lists an older answer (C) as a distractor that reflects the prior birth-cohort-based recommendation. More importantly, the current USPSTF recommendation specifies ages 18-79, not a vague '<80,' and students should know the precise current language. The question could reinforce imprecise understanding of the current guideline.

Q170

Current ACOG and ADA guidelines for gestational diabetes management have evolved. While insulin remains the preferred pharmacologic agent, metformin is now considered an acceptable alternative (not just second-line) when insulin is refused or not accessible, per 2023 ACOG and ADA updates. Additionally, glyburide (choice C) is no longer recommended as a first-line pharmacologic option due to concerns about neonatal hypoglycemia and higher rates of adverse outcomes — a student might not understand why glyburide is wrong for the right current reasons. The framing of the question around 'guidelines from the American...' (truncated) may reference older ACOG guidance. The answer (insulin) is still correct, but the question context around metformin vs. insulin distinctions could mislead students about current practice where metformin has a more accepted role.

Q175

Current guidance on contraception in perimenopausal women has evolved. Combined hormonal contraceptives containing estrogen are generally recommended to be discontinued at age 50-55 or transitioned to progestin-only or non-hormonal methods due to increasing cardiovascular and VTE risk with age. Simply continuing combined OCP in a 45-year-old without reassessment of cardiovascular risk factors and offering alternatives may not reflect current best practice per ACOG and CDC MEC 2024 guidance, where age ≥ 40 raises the risk category for combined hormonal contraceptives.

Q179

For NAFLD/MASLD, while weight loss remains important, GLP-1 receptor agonists (semaglutide) and the FXR agonist resmetirom (Rezdiffra, FDA-approved March 2024) are now first-line pharmacologic treatments for MASH with fibrosis. The question may mislead students about the current preferred pharmacologic options, and GLP-1 analogs (choice B) are now a major recommended treatment — marking it wrong would contradict current 2024 guidelines.

Q183

Varenicline (Chantix) had an FDA black box warning for neuropsychiatric effects that was removed in 2016, but more importantly, the branded Chantix was discontinued in 2021 due to nitrosamine contamination; generic varenicline is now available. More critically, for a post-ACS patient, current 2022+ guidelines (including ACC/AHA) strongly support varenicline as first-line and this remains correct, but the question context about ACS and varenicline safety has evolved — the EAGLES trial and subsequent data confirmed safety. The answer remains correct but the drug name/brand situation may confuse students. However, the larger concern is that combination NRT or cytisine are now also endorsed; varenicline remains first-line so this may not need flagging on that basis alone. Flagging primarily because Chantix (brand) is discontinued and the ACS + varenicline safety framing has updated context.

Q185

The question about contrast-induced nephropathy prevention has evolved significantly. Current evidence (post-2018 AMACING, PRESERVE trials) and updated ACR/NKF guidelines no longer support routine discontinuation of metformin before contrast in patients with eGFR >30 , and the emphasis on NSAID discontinuation specifically in this context has changed. The correct answer of discontinuing naproxen (an NSAID that reduces renal perfusion) is still reasonable, but the question stem implies metformin management is not the concern — current guidelines would flag metformin discontinuation as the primary consideration, making this question potentially misleading about current contrast nephropathy prevention practice.

Q187

The recommended initial duration of bisphosphonate therapy for osteoporosis has nuance that has been updated. Current guidelines (ASBMR, Endocrine Society) suggest 5 years for oral bisphosphonates is appropriate for lower-risk patients, but the framing of a drug holiday and re-assessment criteria has been updated. For a 65-year-old with newly diagnosed osteoporosis, 5 years remains acceptable but current guidance emphasizes reassessment and continuation beyond 5 years for high-risk patients. The answer of 5 years could teach students a fixed endpoint when current practice emphasizes individualized reassessment, potentially leading to premature discontinuation in high-risk patients.